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(متوفرة فقط بي دي اف ولا توجد في المكتبات)

TOXICOLOGY

MCQ

AL HARRIF

ممنوع مشاركتها علي الجروبات او ارسالها لأحد

لطلب المذكرة تواصل مع 01112821905



للتواصل 01112821905

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للتواصل تيليجرام 01112821905

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General toxicology

1- which of the following is safe for emptying the stomach? {Mid2025}

A- warm salty saline B- Apomorphine C- syrup ipecac D- mechanical

2-Cathartics are contraindicated in the following? {Final2025}

A-Ileus and intestinal obstruction B-Hypertension

3- We can use activated charcoal in {Mid2025}

A- lithium B- aspirin C- Strychnine D- benzene

4-Emesis is contraindicated in cases of : {Formative2024}

a) Corrosive b) Coma c) Absent gag reflex d) All of above

5- Absolute contraindication of gastric lavage? {Mid2024}

A. Sulphuric acid B. Child below 6 years C. Hydrocarbons D. Foaming agents

6- Chemical form used in induction of emesis? {Mid2024}

A. Magnesium sulphate B. Sorbitol C. Magnesium citrate D. Serum ipecac

7- Factor related to poison which affect toxicity? {Mid2024}

A. Quantity B. Route of administration

C. Condition of stomach D. General health condition

8- All of the followings are Physiological antidotes except ?.. {Mid2024}

A. Naloxone in opium toxicity B. Barbiturates in strychnine

C. Atropine in organophosphorus poisoning

D. Potassium permanganate for oxidizable poisons as alkaloids

9-One of the following is used as an extracorporeal way for poison elimination as an adsorbant? {Final2025}

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A-Hemoperfusion B-Hemodialysis C-Peritoneal dialysis D-Plasma pharesis

10- All of the following drugs eliminated by forced alkaline diuresis EXCEPT {Final2023}

A- salicylate B- phenobarbital C- lithium D- strychnine

11- Activated charcoal considered as: {Final2021}{Qena}

a. Physical antidote, b. Chemical antidote

c. Pharmacological antidote d. Serological antidote.

12-Gut dialysis refers to: {Mid2021}

a. Multiple doses activated charcoal. b. Gastric lavage

c. Whole bowel irrigation d. None of the above

13- A Policemen arrested a young male with a history of ingestion of drug packets while being arrested. The best method of gastric decontamination in this situation is {Final2022}

A- Gastric lavage B- Whole bowel irrigation C- Syrup of ipecac D- Activated charcoal

14- Choose the most correct statement concerning forced diuresis: {Final2021}{Qena}

a. It is effective in substances that are excreted mainly by kidneys.

b. It is effective in substances with low volume distribution.

c. It is contraindicated in cases of heart failure.

d. All of the above are correct statements.

15-Activated charcoal {Formative2023}

a) In paralytic ileus it should not be given b) May be absorbed by GIT causing toxicity

c) Cathartics may be given to enhance its effect d) It is effective in methanol toxicity

16-The first line of management of acute poisoning is {قديم}{Formative2023}

a) Identify the type of poisoning b) Using antidote

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c) Elimination of poison d) Supportive care for vital signs

17- Antidote of oxalic acid is {Final2021}{Formative2023}

a. Potassium permanganate b. Calcium oxalate

c. Calcium gluconate d. Na bicarbonate

18- 50 years old comatosed male patient came to hospital with history of ingestion unknown substance. Which of the following shouldn't be given to him? {Mid2022}

A- Desferrioxamine B- Dextrose C- Thiamine D- Naloxone

19-Magnesium sulphate is contraindicated in {Mid2023}

a) sinusitis b) hypertension c) renal failure D)None of above

20-Gastric lavage is contraindicated in {Mid2023}

a) Iron b) Corrosive c) Acetaminophen d) Aspirin

21- First step in dealing with intoxicated patient {Mid2024}

A. Airway Breathing Circulation B. Hemodialysis

C. Detailed history D. Gastric lavage

22-The best method for gastric decontamination in a comatosed patient is: {Formative2022}

a) Gastric lavage with cuffed endotracheal tube b) Apomorphine

c) Gastric lavage d) Syrup of ipecac

23-Gasteric lavage is contraindicated in the following poisoning except {Formative2022}

a) aspirin b) sulphuric acid c) oxalic acid d) paracetamol

24-The best emetic by mouth is: {Formative2022}

a) apomorphine b) mechanical emesis. c) syrup epicac d) salt solution

25-The term "Gastrointestinal dialysis" refers to: {Formative2025}{Mid2024}

a) Peritoneal dialysis b) Hemodialysis

c) Alkaline diuresis d) Multiple doses of activated charcoal

26- Activated charcoal is contraindicated in ingestion of {Mid2022}{Final2022} {Formative2025}

A- Acetaminophen B- Aspirin C- Iron D- Datura alba seeds

27- All of the following eliminated by multiple doses of activated charcoal EXCEPT {Final2023}

A- methanol B- theophylline C- digoxin D- phenobarbital

28-The first line of treatment in coma is: {Formative2022}

a) Taking history b) Hemodialysis

c) Safe guard respiration and circulation d) Gastric lavage

29- The first step of management in emergency- of poisoned shocked patient is: {Final2021}

a. Evacuation of the stomach b. Insertion of peripheral intravenous line

c. Give antidote. d. Toxicological screening

30-Activated charcoal is an effective gastric decontamination agent because it: {Merit}

a. Adsorbs and binds toxins, preventing their absorption.

b. Stimulates peristalsis, aiding in toxin removal

c. Reduces stomach acid production

d. Neutralizes acidic substances in the stomach

31-Gastric decontamination is a medical procedure used to {Merit}

a. Induce vomiting in cases of overeating

b. Stimulate appetite in patients with eating disorders

c. Promote digestion in patients with gastric ulcers

d. Remove toxic substances from the stomach

32-Which one of the follwing is contraindicated in hyrocarbon toxicity? {Merit}

a. Antiemetics b. O2 supply c. Antibiotics d. Emesis

33-Pink urine may occur in: {مهم}

a. Phenothiazine b. haemoglobinuria c. Myoglobinuria d. All of the above

34-Which cause of increase bone density: {مهم}

a. Hypervitaminosis A,K in child b. Fluorosis in adult and child

c. Hyper vitaminosis A,D in child d. B&C

35-Which cause of diminished bone density: {مهم}

a. Alcoholism b. Steroids c. Hyper vitaminosis D in adult d. All of above

36-Which is contraindicated of emesis: {مهم}

a. Convulsants, corrosive b. Sharp object, foaming agent

c. Hydrocarbon d. All of above

37-PEG-ELS commonly use in: {مهم}

a. Activated charcoal b. Cathartics c. Whole bowel irrigation d. All true

38-Alkaline diuresis which is useful in: {مهم}

a. Salicylates toxicity b. Phenobarbital toxicity c. Lithium toxicity d. All true

39-Which toxin has significant enterohebatic circulation, that we use in toxicity case MDAC: {مهم}

a. Digoxin b. Phenobarbital c. Theophylline d. All of the above

40-Potassium permanganate (for oxidisable poisons) is: {مهم}

a. Mechanical antidote b. Chemical antidote

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c. Physiological antidote d. Serological antidote

41- 35 female came to emergency department was comatosed and cyanosed with a history of suicidal attempts with organophosphorus , 20 tablets paracetamol 500 mg , 30 tablets folic acid what the most important line in treatment {Qena}

a) Atropine b) N-Acetylcysteine c) Resuscitation d) Obidoxime

42- A 43 year old female patient came to the ER with disturbed conscious level and cyanosis with history of suicidal attempt by alcohol, 20 tablets of paracetamol 1 gm and 3 tablets of tramadol. The most important line of treatment in this situation: {Qena}

a. Atropine b. Naloxone c. N-acetylcystein d. Resuscitation

43- Multiple doses of activated charcoal is recommended in the following cases except: {قديم} {Qena}

A- Poisons having enterhepatic circulation B- Poisons having long half life

C- Poisons having anticholinergic action D- Long acting sustained release preparations

44-A toxicant produced by an animal is called {Qena}

a. toxin b. bictoxin c. venom d. poison e. mycotoxin

45- the following is contraindication of activated charcoal {Qena}

a)Corrossives b)Hydrocarbons c)Paralytic ileus d)All of the above

46-Heavy metals are characterized by all of the following except : {Qena}

A. They cause local action only

B. They cause acute and chronic toxicity

C. They have great affinity to SH group containing enzymes

D.They are excreted mainly through the kidney

47-One of the ideal homicidal poisons is {Qena}

a- Salicylates b- Carbolic acid c) Arsenic d) Strychnine

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48-presence of fatty food in the stomach always delays the absorption of toxic agents. {Qena}

a. True b. False

49- Apomorphine could be used at home for induction of vomiting. {Qena}

a. True b. False

50-The following are convulsant poisons except: {Qena}

A-strychnine b-cocaine c-morphine d- oxalic acid

51- Salty water is effective as an emetic agent. {Qena}

a. True b. False

52-Dialyzable substances include the following, except: {Qena}

a- Barbiturates b- Opiates c- Salicylates d- Alcohols

53-safe and effective method emptying of the stomach is: {Qena}

a-salty hot water. b-Apomorphine

c-Syrup of ipecac. d- Touching back of the tongue

54-The best method to avoid aspiration of fluids during gastric lavage in a comatose patient is by: {Qena}

a. Putting the head of the patient at a lower level than his feet

b. Putting the patient in the left lateral position

c. Introduction of a cuffed endotracheal tube before lavage

d. Continuous suction of the fluid from the trachea

55-In patients with white eschars, carpopedal spasm, oliguria, white crystals in urine, the life saving measure is {Qena}{kasr}

a.Emesis. b.Glycerin. c.Calcium gluconate. d.Diazepam

56- remove of drug packet from body? {Qena}

A. Whole bowel irrigation B. Gastric lavage C. Emesis D. Active charcol

57-cyanide poisoning is ineffective in cases of: {Qena}

a.Taking low dose b.gas form of cyanide c. hyperacidity d.achloridria.

58-Syrup of epicac is given: {kasr}

a. IV b. IM. c. Orally d. SC.

59-Emesis is contraindicated in all the following conditions, except: {kasr}

a. CNS stimulation. b. Unconscious patients

c. Esophageal varices. d. Infant is over 6 months

60- Syrup of epicac stimulates emesis within: {kasr}

a. 10 minutes. b. 30 minutes. c. 45 minutes. d. 60 minutes.

61-Induction of vomiting is contraindicated in all the following poisons, except: {kasr}

a. Kerosene b. Carbolic acid. c. Digitalis. d. Nitric acid

62-Synergism means: {kasr}

a. The chemicals add to the effect of each other

b. One chemical exaggerates the effect of another

c. One chemical diminishes the effect of another. d. Abnormal response to drugs.

63-Achlorhydria prevents toxicity of {kasr}{Qena}

a. K cyanide. b. Arsenic. c. Alcohol d. Digitalis

64-BAL should be given only by: {kasr}

a. IV. b. IM. c. IP. d. Orally.

65- When sulphonamide is given to a patient with G-6PD. It leads to hemolytic anemia. This response is: {kasr}

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a. Hypersensitivity b. Idiosyncrasy c. Tolerance d. Synergism.

66-Addicts can take large doses of abused drugs without serious effects due to: {kasr}

a. Tolerance b. Addiction. c. Idiosyncrasy. d. Allergy

67-All the following poisons have local &remote actions, except: {kasr}

a. Nitric acid. b. Carbolic acid. c. Arsenic. d. Oxalic acid.

68-Poisons with remote action only include all the following, except: {kasr}

a. Atropine b. Strychnine c. Morphine d. Oxalic acid

69-Gut dialysis refers to {kasr}

a. Activated charcoal b. MDAC c. Cathartics. d. Whole bowel irrigation.

70-Alkaline diuresis enhances the excretion of: {kasr}

a. Phenol. b. Cyanide c. Amphetamine. d. Aspirin.

71-Poorly adsorbed poisons by activated charcoal include: {kasr}

a. H2SO4 b. Mercury. c. Alcohol d. All of the Above

72-Ion trapping is: {kasr}

a. Increasing the glomerular filtration rate. b. Changing PH Of urine.

c. Irrigation of the GIT with non absorbable solution d. None of the above.

73-Alkalinization of urine hastens the excretion of: {kasr}

a. Aspirin. b. Barbiturates. c. Carbolic acid d. Both A & B

74-Dialyzable drug must have: {kasr}

a. Molecular weight > 500 Daltons. b. Low lipid solubility.

c. High protein binding d. None of the above.

75-Removal of the poison from the blood by increasing the glomerular filtration rate is: {kasr}

a. Forced diuresis. b. lon trapping c. Hemoperfusion. d.Hemodialysis.

76-One of the following methods of extracorporeal person. removal depends on adsorption: {kasr}

a. Peritoneal dialysis b. Hemodialysis c. Hemoperfusion d. Plasmapheresis

77-Non dialyzable substances haves {kasr}

a. High protein binding b. Low water solubility,

c. High lipid solubility. d. All of the above

78-Dicobalt EDTA is used to treat poisoning of: {kasr}

a. Digitalis. b. Lead. c. Cyanide. d. Mercury

79-Hemoperfusion is not effective if the toxic agent is: {kasr}

a. Water insoluble b. Lipid soluble.

c. Not adsorbable by charcoal. d. Bound to protein

80-Multiple dose activated charcoal may be effective in removing each of the following intoxications, except: {kasr}

a. Barbiturates b. Salicylates c. Digitalis d. Lithium

81-Coma cocktail consists of dextrose, naloxone in addition to: {kasr}

a. Vitamin B1 b. Vitamin B6 c. Vitamin B12 d. None of the above

82-An intoxication caused by tricyclic antidepressants is most similar to the one caused by: {kasr}

a. Amphetamines b. Atropine

c. Barbiturases d. The withdrawal of barbiturates

83-Hemodialysis is a line of treatment of poisoning with: {kasr}

a. Propranolol & TCA's b. Chlorpromazine

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c. Ca channel blockers. d. Lithium & theophylline

84-Gastric lavage may be allowed in comatose patient. {kasr}

a. true b. false

85-Apomorphine is superior to syrup of epicac for induction of vomiting. {kasr}

a. true b. false

86-Safe & effective emptying of stomach is done by: {kasr}

a. Salty hot water. b. Apomorphine.

c. Syrup of epicac. d. Touching the back of tongue

87-Activated charcoal is effective in caustic ingestion. {kasr}

a. true b. false

88-Castor oil is used as a cathartic in parathion poisoning. {kasr}

a. true b. false

89-Gut dialysis refers to MDAC. {kasr}

a. true b. false

90-Changing the pH of urine enhances the elimination of some drugs. {kasr}

a. true b. false

91-BAL is a chelator for heavy metals. {kasr}

a. true b. false

92-BAL is used to chelate iron. {kasr}

a. true b. false

93-Activated charcoal is effective in cases of ethanol intoxication. {kasr}

a. true b. false

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94-MDAC is used in treatment of short-acting barbiturate overdoses. {kasr}

a. true b. false

95-Barbiturates have CNS depressant effect. {kasr}

a. true b. false

96-Amphetamine is a strong CNS stimulant. {kasr}

a. true b. false

97-All these drugs cause slow gastric motility except: {kasr}{قديم}

a. Barbiturates b. Atropine c. Cocaine. d. Morphine

98-Whole bowel Irrigation is indicated in: {kasr}

a. Body packers of illicit drugs b. Poorly adsorbed drugs by charcoal.

c. Slow release preparations. d. All of the above.

99-Common use cathartics is:

a. Hydrogen sulphate b. Magnesium chloride c. Sorbetal d. Magnesium sulphate

100-We use radiopacity in all of the following except:

a. Lead b. Iron c. Package (cocaine, heroin) d. Chloral hydrate

101-What is the true component of coma cocktail:

a. 5% dextrose, 3mg naloxone, thiamine b. 50% dextrose, 2mg naltrexone, folic acid

c. 15% dextrose, 2mg naltrexone, thiamine d. 50% dextrose, 2mg naloxone, vitamin B1

102- Multiple dose activated charcoal is given in digoxin toxicity because:

a. It has an enterohepatic circulation. b. It forms concretions.

c. It is re-excreted in the stomach. d. It is re-excreted in the intestine.

103- Emesis must be induced by:

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a. Tincture of ipecac. b. Concentrated salt solution.

c. Syrup of ipecac. d. Sodium bicarbonate solution.

104- The first line of management of deep unknown coma is:

a. Immediate gastric lavage & glucose. b. Antidote treatment & glucose.

c. Maintenance of patent airway, oxygen & glucose.

d. Emesis with syrup of: ipecac then glucose.

105- In absence of syrup ipecac emesis can be induced by:

a. Mechanical stimulation of the oropharynx. b. Tincture ipecac.

c. Salt solution. d. Metoclopramide.

106- Emesis is avoided in cases of:

a- Convulsions. b- Ingestion of sharp objects. c- Pregnancy.

d- Coma. e- All of the above

107- Multiple dose activated charcoal is very effective for adsorption of one of the followings:

a- Iron. b- Lithium. c- Salicylate. d- Lead.

108-Activated charcoal is so important material used in detoxification of the toxin patient, what is the mechanism of action of activated charcoal?

a. Absorb the toxin b. Help the kidney to excrete the toxin rapidly

c. Adsorb the toxin d. Help body to metabolize the toxin rapidly

109-Two addictive patients 25 and 27 years old came to the hospital with manifestation of overdose toxicity, after analysis both were positive for heroin. Patient one was more toxic and died after 2 days, patient 2 did not deteriorate and progressively improved with treatment. Doctors explained that the route of administration of the heroin was different in both patients. What is the expected route of administration in both patients?

a. Patient 1 oral and patient 2 subcutaneous

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b. Patient 1 intravenous and patient 2 inhalation

c. Patient 1 intradermal and patient 2 subcutaneous

d. Patient 1 by inhalation and patient 2 intravenous

110-Method of elimination:

a. Gastrointestinal dialysis b. Forced alkaline diuresis c. Dialysis d. All of the above

111-In which situation would there be an increase in the toxic effect of a substance due to patient-related factors?

a. Male gender b. Age extremes c. Full stomach d. Healthy liver function

112-BAL can be used in the treatment of poisoning with:

a. Iron. b. Arsenic. c. Mercury d. a & b. e. b&c.

113-Desferrioxamine mesylate is used in treatment of poisoning with:

a. iron. b. Arsenic. c. Mercury d. Lead. e. Ethyl alcohol.

114-Emesis is avoided in cases of:

a- Convulsions b- Ingestion of sharp objects. c- Pregnancy.

d- Coma e- All of the above.

115-The best method to avoid aspiration of the fluid during gastric lavage in a comatose patient is by:

a- Putting the head of the patient at lower level and at one side.

b- Putting the patient in lateral position.

c- Introduction of cuffed endotracheal tube before lavage.

d- Continuous suction of the fluid from the trachea.

e- None of the above.

116- Single dose of activated charcoal for adsorption of the poison is:

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a- One mg/kg b.w. b- One gm/kg b.w. c-10 mg/kg b.w.

d-10 gm/kg b.w. e- 100 mg/kg b.w.

117-Chelators unite with the absorbed poison to form:

a- Less soluble and less toxic substance.
b- Less soluble and non excreted substance.

c- More soluble and more toxic substance.

d- More soluble and less toxic easily excreted substance.

e- More toxic and non excreted substance.

118-In haemodialysis, all of the followings are true Except:

a- It is suitable for high plasma poison concentration.

b- It is suitable for poisons having small molecular weight.

c- It is suitable for poisons having high protein binding.

d- It is effective for dialysis of alcohol and aspirin.

e- It is suitable for water soluble poisons.

119-All of the followings antidotes are given parenterlly Except:

a- Naloxone. b- Atropine. c- EDTA.

d- Penicillamine and DMSA. e- Hydroxy-cobalamine.

120-In poisoning with sulphuric acid the tissues are stained:

a. Black. b. Gray. c. Blue d. Yellow. e. Pink.

121-Hypothermia is characteristic in poisoning by:

a. Tricyclic antidepressants. b. Atropine. c. Morphine d. hyoscine

122-An Important antidote in the treatment of cyanide poisoning is:

a. Desferrioxamine mesylate (Desferal). b. Dicobaltedetate (Kelocyanor).

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c. Ethanol. d. Atropine

123-Alkalinization of urine is not effective with the following poisons Except

a. Barbiturates b. Morphine. c. Corrosives

d. Lead. e. Benzodiazepines.

124-Activated charcoal is used in treatment of acute poisoning with:

a. Ethanol. b. Methanol. c. Kerosene.

d. All of the above e. None of the above.

125-In achlorhydria, which poison is not fatal?

a. Iron. b. Arsenic. c. Potassium cyanide.

d. Hydrocyanic acid. e. Opiates.

126-What is tolerance?

a. Reaching the effect of large dose after multiple small doses.

b. Small harmless doses of drugs produce severe symptoms.

c. Large dose of some poisons may not cause toxicity if previously given in repeated small doses.

d. When person has unusual reaction to certain drug.

e. Characteristic illness occur on withdrawal of the drug.

127-What is idiosyncrasy?

a. Reaching the effect of large dose after multiple small doses.

b. Small harmless doses of drugs produce severe symptoms.

c. Large dose of some poisons may not cause toxicity if previously given in repeated small doses.

d. When person has unusual reaction to certain drug.

e. Characteristic illness occur on withdrawal of the drug.

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128-All the followings are emergency measures in treating poisons Except

a. Maintain clear airway. b. Maintain breathing. c. Fluid diuresis.

d. Maintain fluid and electrolyte balance. c. Assess neurological status.

129-Antidotes include all Except:

a. Antagonists. b. Competitors. c. Hypoallergenics. d. Chelators.

130-A chelator may be:

a. Naloxone. b. DMSA c. Ethyl alcohol.

d. Nallorphine. e. Atropine.

131-A chelator may be:

a.Naloxone. b. BAL. c. Ethyl alcohol.

d. Nallorphine. e. Atropine.

132-A chelator may be:

a. Naloxone. b. Dimercaprol c. Ethyl alcohol.

d. Nallorphine. e. Atropine.

133-A chelator may be:

a. Naloxone b. Penicillamine. c. Ethyl alcohol. d. Nallorphine. e. Atropine.

134-A chelator may be:

a. Naloxone. b. Kelocyanor. c. Ethyl alcohol. d. Nallorphine e. Atropine.

135-An Important antidote in the treatment of cyanide poisoning is:

a. Desferrioxamine mesylate (Desferal). b. Dicobaltedetate (Kelocyanor).

c. Ethanol. d. Atropine

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1. C	2. A	3. B	4. D	5. A
6. D	7. A	8. D	9. A	10.D
11.A	12.A	13.B	14.D	15.A
16.D	17.C	18.A	19.C	20.B
21.A	22.A	23.C	24.C	25.D
26.C	27.A	28.C	29.B	30.A
31.D	32.D	33.D	34.D	35.D
36.D	37.C	38.D	39.D	40.B
41.C	42.D	43.C	44.C	45.D
46.A	47.C	48.B	49.B	50.C
51.B	52.B	53.C	54.C	55.C
56.A	57.D	58.C	59.D	60.B
61.C	62.B	63.A	64.B	65.B
66.A	67.A	68.D	69.B	70.D
71.D	72.B	73.D	74.B	75.A
76.C	77.D	78.C	79.C	80.D
81.A	82.B	83.D	84.A	85.B
86.C	87.B	88.B	89.A	90.A
91.A	92.B	93.B	94.B	95.A
96.A	97.C	98.D	99.D	100. A
101. D	102. A	103. C	104. C	105. A
106. E	107. C	108. C	109. B	110. D
111. B	112. E	113. A	114. E	115. C
116. B	117. D	118. C	119. D	120. A
121. C	122. B	123. A	124. E	125. C
126. C	127. D	128. C	129. C	130. B
131. B	132. B	133. B	134. B	135.

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Corrosive

1-Hypocalcemia is a feature of poisoning with: {Mid2024}{Formative2025}

a) Sulfuric acid b) Oxalic acid c)Carbolic acid d) Hydrochloric acid

2-Morphine used in treatment of {Final2025}

A-Phenol acid B-Oxalic acid C-Sulphuric acid D-Barbiturates

3- gastric lavage by alcohol is used in toxicity of {Mid2025}

A- sulfuric acid B- phenol C- oxalic acid D- alkaline potash

4-After 15minutes, a patient with oxalic acid toxicity can die from {Final2025}{kasr}

A-Neurogenic shock B-Central asphyxia C-Peripheral asphyxia D-Renal failure

5-In sulfuric acid poisoning treatment : {Formative2024}

a) Anticonvulsant are given iv b) Respiratory stimulants are life saving

c) Emesis contraindicated d) Renal dialysis is preferred

6- Sulphuric acid act by {Final2023}

A- Coagulative necrosis B- Liquefactive necrosis

7-Methemoglobinemia may occur after poisoning with: {Formative2022}

a) Phenol b) Sulfuric acid c) Morphine d) Hydrochloric acid

8- Green color of urine in {Mid2024}

A. Phenol B. Hydrochloric acid C. Oxalic acid D. Nitric acid

9- Methemoglobinemia is caused by {Mid2024}

A. Sodium hydroxide B. Carbolic acid C. Sulphuric acid D. Hydrochloric acid

10- A 17-yrar old boy has ingested phenol from the cleaning substances at home. {Mid2023}

a) Local b) Kidney c) heart d) all of the above

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11- The best time to perform fiberoptic endoscopy for grading esophageal lesion due to strong alkali ingestion: {Mid2021}{Formative2024}

a. In the first 24 hours b. After one week c. After two weeks d. After 1 Month

12-In Carbolic acid toxicity, dark green color of urine is due to {Formative2023}{Qena}

a. Hydrosulfate b. Sodium bicarbonate c. Hydroquinone d. Calcium oxalate

13- Antidote of oxalic acid is {Final2021}

a. Potassium permanganate b. Calcium oxalate

c. Calcium gluconate d. Na bicarbonate

14- Diarrhea may occur in toxicity of {Mid2022}{kasr}

A- Sulphoric acid B- Alkaline potash C- Hydrochloric acid D- Nitric acid

15- Liquifactive necrosis is the action of {Mid2025}{kasr}

A- Sulphuric acid B- Alkaline potash C- Hydrochloric acid D- Nitric acid

16-A 17-year-old boy has ingested sulfuric acid, which is the mode of action? {Mid2023}

a) coagulative necrosis b) liquefactive necrosis

17- Mechanism of action of hydrochloric acid is {Mid2024}

A. Coagulative necrosis B. Liquefactive necrosis C. Anticholenirgic

18- Color of urine in carboluria is {Final2023}

A- yellow B- brown C- green

19-A 17-year old boy has ingested HCL, which of the following should be used? {Mid2023}

a) syrup of ipecac b) activated charcoal c) milk d) gastric lavage

20-All the following are characteristic of alkaline ingestion except: {Qena}

A) Liquefactive necrosis B) Coagulative necrosis

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C) Denaturation D) Saponification of fats

21-Which one of the following treatment is important for decrease fibrosis in a case of corrosive inury? {Merit}

a. Antibiotics b. Antiemetics c. Laxatives d. Steroids

22-The main role of steroids in management of acute inorganic corrosives is {Qena}

a-pain killer b-prevent fibrosis c-prevent bleeding d-prevent ulceration

23- Child 2 years ... his urine changed to dark green? {Qena}

a. Toxicity with carbolic acid

24- steroids indication for which of the following {Qena} تاكد

a) prevent fibrosis b) prevent bronchospasm

c) prevent edema&stridor d) prevent infection

25- Yellow corrosion around the mouth is seen ingestion of: {Qena}

a) Sulphuric acid b) Nitric acid c) Hydrochloric acid d) Hydofluroic acid

26-After 4 weeks, child with corrosive ingestion may die from: {Qena}

a. Malnutrition. b. Shock. c. Hematemesis. d. Electrolyte imbalance

27-All the following are true about corrosive poisoning except {Qena}

A. It is usually accidental in children

B. On ingestion commonly there is vomiting, diarrhea and severe burning pain

C. On inhalation, onset of respiratory symptoms is early

D. Early neutralization is important

28-Select the wrong statement regarding corrosive poisoning: {Qena}

a. Chemical antidote is indicated for neutralization.

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b. Acids change hemoglobin to acid hematin.

C. Most of the cases occur accidentally in children.

d. In some cases, poisoning associated with hypocalcemia.

29-Which is used in treatment of corrosive poisoning: {Qena}

a Gastric lavage b. Emesis c. Cathartics d. Demulcents

30- Convulsions of oxalic acid poisoning are best treated with {قديم}{Qena}

A- Diazepam B- MgSO4 C- Calcium gluconate D- None of the above

31- Why is gastric lavage generally contraindicated in cases of corrosive poisoning {Qena}

A- due to the risk of perforation

B- Insufficient after 1 hour

C- It may increase systemic absorption of the corrosive substance

32- Causes of death in corrosive in first 24 h except: {Qena}

A. Shock B. Laryngeal edema C. Exhaustion D. Perforation

33- within 4 weeks of children ingestion of potash, the suitable cause of death is: {Qena}

A. Hematemesis B. Shock C. Malnutrition D. Electrolyte disturbance

34-Phenol is dissolved by: {kasr}

a. MgSO4 b. Egg white c. KMO4, 1/5000 d. Ethanol 10%

35-All the following could be the cause of death in alkaline potash poisoning except: {kasr}

a. Neurogenic shock. b. Hypovolemic shock.

c. Laryngeal edema d. Liver failure.

36-In alkaline potash toxicity, cachexia can result from: {kasr}

a. Perforation of the stomach. b. Stricture of the oesphagus.

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c. Dehydration. d. Shock

37-Eschars of nitric acid toxicity is characterized by: {kasr}

a. White in color. b. Black in color. c. Yellow in color d. None of the above.

38-2 years old child swallowed diluted alkaline potash, the best fluid to treat this condition is: {kasr}

a. Saline. b. Milk. c. H_2O. d. Carbonated beverages.

39-In carbolic acid toxicity, the urine turns green on exposure to air due to: {kasr}

a. Sodium bicarbonate. b. Potassium permanganate.

c. Hydroquinone. d. Hydrosulfate.

40-After 4 hours, a patient with carbolic acid toxicity can die from: {kasr}

a. Hypovolemic shock. b. Central asphyxia

c. Peripheral asphyxia. d. Renal failure.

41-Gastric lavage is indicated in: {kasr}

a. Sulphuric acid toxicity. b. Alkaline potash toxicity.

c. Hydrochloric acid toxicity. d. Carbolic acid toxicity.

42-In carbolic acid toxicity, ethanol is used as local antidote because it: {kasr}

a. Precipitates phenol. b. Dissolves phenol.

c. Neutralizes phenol d. Adsorbs phenol.

43-Cardiac arrest in diastole can occur in: {kasr}

a. Inorganic corrosives. b. Carbolic acid toxicity.

c. Oxalic acid toxicity d. All of the above.

44-wash with alcohol can be used in treatment of: {kasr}

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a. Phenol. b. Oxalic acid. c. Nitric acid. d. Sulphuric acid.

45-The following drugs are corrosives, except: {kasr}

a. H_2SO_4 b. Alkaline potash C. KCN d. Iead

46-Regarding corrosives: {kasr}

a. With alkali injury, tissues soften liquefy, facilitating penetration of the alkali to deeper tissue layers.

b. After alkali ingestion, the first and most common site of injury is the squamous epithelium of the esophagus.

c. Acid-induced gastric burns are very common, especially in the distal portion of the stomach.

d. All of the above is true

47-Neutralization of corrosives by either weak acids or alkalies is obsolete. {kasr}

a. true b. false

48-Stomach wash is indicated in treatment of phenol toxicity. {kasr}

a. true b. false

49-Diarrhea is a common symptom after sulphuric acid toxicity. {kasr}

a. true b. false

50-Dilution with water is used in treatment of alkaline potash toxicity. {kasr}

a. true b. false

51-Neutralization with weak alkali is used in the treatment of sulphuric acid toxicity. {kasr}

a. true b. false

52-Corticosteroids are used in the treatment of inorganic corrosives. {kasr}

a. true b. false

53-Methylene blue can be used in the treatment of carbolic acid toxicity. {kasr}

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a. true b. false

54-Select the correct statement regarding corrosives poisoning treatment

a. Dilution with water and/or milk b. Neutralization

c. gastric lavage d. Activated charcoal

55-Regarding Caustic alkalis, Which one of the following statements is correct

a. Cause Liquefactive necrosis b. Cause Coagulative necrosis

c. Cause Oxidation d. Can be found in Nail cauterization, disinfectants & insecticides

56-Oxalic acid' remote actions include

a. Acute hemolysis b. Elevated liver enzymes.

c. Arrhythmias, hypocalcemia and tetany d. Esophageal perforation

57-Oliguria, loin pain & greenish discoloration of urine are manifestations of

a. Hydrochloric acid toxicity b. Carbolic acid toxicity.

c. Caustic soda (NaOH) toxicity. d. Bleaching agents toxicity

58-In toxic alkaline base ingestion injury pattern, one of the following is true:

a) Liquifactive necrosis. b) Coagulative necrosis.

c) Gastric perforation. d) Eschars formation.

59-In carbolic acid poisoning, urine is:

a. Orange color b. Dark and when exposed to air turn green.

c. Dark and when exposed to air turn yellow. d. Normal (amber yellow).

60-All of the followings are correct Except:

a- Mineral acids include sulphuric nitric and hydrochloric acid.

b- Organic acids include phenol, oxalic acid and acetic acid

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c- Vegetable acid include hydrocyanic acid

d- Alkalis include caustic potash, ammonium hydroxide and mercuric chloride.

61-Corrosive alkalies include:

a- Sodium hydroxide. b- Ammonium chloride. c- Potassium iodide.

d- Oxalic acid. e-Carbolic acid.

62-Poisoning with corrosive alkalies:

a- Mainly accidental. b- Mainly suicidal. c- Mainly homicidal.

d- Should be treated by induction of emesis.

e- Should be treated by administration of activated charcoal.

63-The ingestion of corrosive inorganic acids produces the following Except:

a- Oliguria b- Diarrhea. c- Haematemesis.

d- Shock. e-Oedema of the glottis.

64-Due to ingestion of caustic potash, one of the following manifestations is absent:

a- Oedema of the glottis. b- Dehydration. c- Constipation.

d- White vomitus. e- Shock.

65-Dehydration is absent in poisoning by:

a- Oxalic acid. b- Acetic acid. c- Potassium hydroxide.

d- Carbolic acid. e- Nitric acid.

66-The best treatment of convulsion due to oxalic acid poisoning is the I.V. infusion of:

a- Diazepam. b- Magnesium sulphate.- c- Morphine sulphate.

d- Short acting barbiturate e- Calcium gluconate.

67-A poison which can be diagnosed by microscopical examination of the urine is:

a- Ammonium hydroxide. b- Oxalic acid. c- Hydrocyanic acid.

d- Nitric acid. e- Caustic potash.

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68-After skin contamination, the patient had coma, miosis and acute nephritis, the poison is:

a- Oxalic acid. b- Nitric acid. c- Hydrocyanic acid.

d- Carbolic acid. e- Ammonium hydroxide.

69-Cardiac toxicity in oxalic acid poisoning is due to relative decrease of:

a- Potassium. b- Calcium. c- Sodium. d- Magnesium. e- Acetylcholine.

70-Asphyxia due to spasm of the respiratory muscles occurs in poisoning by:

a- Ammonium hydroxide. b- Carbolic acid. c- Oxalic acid.

d-Acetic acid. e-Hydrochloric acid.

71-The following poisons have characteristic smell Except:

a- Carbolic acid. b- Hydrocyanic acid. c- Kerosene. d- Oxalic acid..

72-Corrosive alkalies include:

a. Sodium hydroxide. b. Potassium hydroxide. c. Ammonium hydroxide.

d. All of the above. e . None of the above.

73-Vomiting in case of poisoning with corrosive alkalies is:

a. Absent. b. Littic. c Greenish.

d. Slimy containing altered blood e. Brown & contains white crystals.

74-One of the following manifestation is absent in the ingestion of ammonium hydroxide:

a. Oedema of the glottis. b. Dehydration. c. Constipation.

d. Slimy vomitus. e. Shock.

75-Urine becomes dark smoky green colour on exposure to air in case of poisoning with:

a. Hydrocyanic acid. b. Diluted oxalic acid. c. Caustic potash.

d. Carbolic acid e. Hydrochloric acid.

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76-Cardiac toxicity in oxalic acid poisoning is due to decreased serum level of:

a. Potassium. b. Calcium. c. Sodium.

d. Magnesium e. Acetylcholine.

1. B	2. C	3. B	4. C	5. C
6. A	7. A	8. A	9. B	10.D
11.A	12.C	13.C	14.B	15.B
16.A	17.A	18.C	19.C	20.B
21.D	22.B	23.A	24.C	25.B
26.A	27.D	28.A	29.D	30.C
31.A	32.C	33.C	34.D	35.D
36.B	37.C	38.B	39.C	40.B
41.D	42.B	43.C	44.A	45.D
46.D	47.A	48.A	49.B	50.B
51.B	52.A	53.A	54.A	55.A
56.C	57.B	58.A	59.B	60.D
61.A	62.A	63.B	64.C	65.D
66.E	67.B	68.D	69.B	70.C
71.D	72.D	73.D	74.C	75.D
76.B	77.	78.	79.	80.

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Irritant poisons

1-Chronic exposure Causes permanent neurological and developmental distrubance in children? {Final2025}

A-Lead B-Mercury C-Arsenic D-Cadmium

2-Poisoning with Gingivitis, discolouration of gums and lose teeth in {Final2025}

A-Mercury poisoning B-Lead poisoning

C-Iron poisinong D-Cadmium poisoning

3- Desferroximine is antidote of {Final2023}

A- iron B- lead C- Mercury

4- A man is worker in medical equipment industries presented with grey linen on the gum, gingivitis, tremors mostly chronic toxicity of {Mid2024}

A. Mercury B. Lead C. Iron D. Arsenic

5- Specific chelating agent in iron poisoning is {Mid2024}

A. Penicillamine B. Desferrioxamine C. EDTA D. BAL

6- Dysentery is caused by which poison {Final2023}

A- lead B- mercury C- arsenic D- sulphuric acid

7- Regarding iron poisoning, which is TRUE? {Final2022}

A- Activated charcoal is the recommended method of GIT decontamination in the first hour.

B- Has no local GIT irritating effects.

C- Penicillamine is the chelating agent of choice.

D- Desferrioxamine is the chelating agent and when given will produce vine rose colored urine.

8-The main route of iron excretion is: {Mid2021}

a. Urine b. Stool c. Bile d. Saliva

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9- The vine rose color of urine is a characteristic in treatment of: {Final2021}

a. Iron toxicity b. Lead toxicity c. Mercury toxicity d. All of above

10- Kinetic tremors occur in chronic poisoning with {Final2022}{kasr}

A- Arsenic B- Lead C- Mercury D- Antimony

11- ..... is the best for lead chelation in children with a lead level higher than 45µg/dl. {Mid2021}

a. EDTA b. DMSA c. BAL d. Penicillamine

12- As regard lead..... 95% distributed lead is present in: {Final2021}

a. Liver b. Blood c. long bone d. Brain

13- Kinetic tremor is one sign of: {Final2021}

a. Lead poisoning b. Iron poisoning c. Mercury poisoning d. Arsenic poisoning

14- workers in the manufacture of medical thermometers may suffer from {قديم }

A- encephalopathy and blue gum line B- Kinetic tremors C- Mee's line D- phossy jaw

15-Plumbism is due to chronic poisoning with {Formative2022}

a) Arsenic b) Lead c) Copper d) Mercury

16-Peripheral neuritis in plumbism is {Formative2024}

A. Purely motor B. Purely sensory

C. Mixed but more sensory D. Mixed but more motor

17-Which of the following tooth colors indicates lead poisoning (plumbism)? {مهم}

a. Yellow line. b. Green line. c. Blue line. d. Grey line.

18-Grey line on gums in case of poisoning with.:- {مهم}

a. Copper b. Mercury c. Lead d. Arsenic

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19- A child broke a clinical thermometer and swallowed its mercury, the best treatment is: {Qena}

a) Forced diuresis. b) Sodium thiosulphate I.V. c) BAL I.M.

d) All of the above. e) None of the above.

20- The following signs are characteristic of plumbism except: {Qena}

a. pallor. b. blue lines on gums. c. sensory neuritis d. constipation

21- Characteristic for mercury poisoning {Qena}

A. Erethism

22- Vitamin D helps in treatment of poisoning by: {Qena}

a. Arsenic b. Merqury c. Lead d. Iron

23 -Mercurialism is characterized by: {Qena}

a. Gingivitis b. Rice watery diarrhea c. Jaw necrosis d. Optic nerve atrophy

24-Nervous manifestation of lead poisoning is: {Qena}

a.Erethism b.Foot and wrist drop

C. Kinetic tremors D. Tingling and numbness in skin

25-Mercurialism is characterized by all of the following except: {Qena}

A. kinetic tremors b. food and wrist drop c. Erethism D. Gingivitis

26-Triad of chronic mercury toxicity includes the following, except: {Qena}

a. Tremors b. Gingivitis c. Erethism d. pupillary constriction

27-Putrefaction is delayed in cases of poisoning with: {kasr}

a. Opium. b. Arsenic. c. Parathion. d. Carbon monoxide.

28-Arsenic is re-excreted in: {kasr}

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a. Stomach. b. Caecum c. Ascending colon. d. Descending colon.

29-Re-excretion of mercury occurs in: {kasr}

a. Stomach. b. caecum. c. Ascending colon. d. Descending colon.

30-Circle the statement that does not go along with the clinical picture of chronic mercury toxicity: {kasr}

a. Presence of grey line in the gums.

b. Tremors of the muscles of the fingers and tongue.

c. Chronic constipation. d. Psychic problems as anxiety and vague fear

31-Which of the following is not toxic if ingested orally? {kasr}{قديم}

a. Lead paints b. Arsenic salts c. Iron sulphate. d. Elemental mercury.

32-The mechanism of lead induced anemia is: {kasr}

a. Inhibition of erythropoietin synthesis. b. Decreased RBCs survival.

c. Inhibition of heme production d. All of the above.

33-Adverse effects of lead poisoning include all the following except: {kasr}

a. Inhibition of enzymes associated with heme synthesis.

b. Blocks absorption of vitaminB12.

c. Elevation of free erythrocyte protoporphyrin

d. RBCs basophilic stippling

34-Deferoxamine is used in the treatment of poisoning with: {kasr}

a. Arsenic b. iron c. Lead d. Mercury.

35-Which is not true about deleading: {kasr}

a. It is the process of mobilization of lead from bones & removal of lead from body.

b. It should be done cautiously& gradually in hospital.

c. Excessive mobilization of lead may result in acute lead poisoning.

d. None of the above

36-The following but One are characters of heavy metal toxicity {kasr}

a. Delayed onset of symptoms. b. Mild G.I.T. irritation.

c. Cumulative effect d. Disappear by putrefaction

37-Encephalopathy occurs most rapidly in poisoning by leads {kasr}

a. Tetra ethyl. b. Metal c. Arsenate d. Oxides

38- Encephalopathy may be caused by the following poisoning, except: {kasr}

a. Tetraethyl lead. b. CO c. Methanol. d. Phenol

39-Which of the following may be a manifestation of lead poisoning: {kasr}

a. Metabolic acidosis. b. Hypokalemia. c. Cerebral edema. d. Mydriasis.

40-All of the following are classical signs and symptoms of chronic mercury poisoning except: {kasr}

a. Gingivitis b. Anxiety and memory disturbance.

c. Hepatitis. d. Tremors of fingers and tongue.

41-Which of the following statement is not correct regarding chronic lead toxicity? {kasr}

a. It may cause sterility in both sexes

b. It is not likely to cause emotional disturbance or tremors.

c. Abdominal colic is more common with inorganic than organic lead.

d. It carries the same risk to all ages

42-Most of the acute cases of poisoning is due to: {kasr}

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a. Accidental poisoning from chemical sources

b. Homicidal poisoning from chemical or plant sources.

c. Accidental poisoning from animal and plant sources.

d. All of the above occur at the same frequency.

43-In acute iron poisoning, stage III is not characterized by: {kasr}

a. Metabolic acidosis. b. Hypotension. c. Cyanosis. d. Fever.

44-A middle aged male working in a lead factory, with a full set of denture may suffer from the following except: {kasr}

a. Constipation. b. Blue line in gums. c. Wrist drop d. Impotence

45-Lead microcytic hypochromic anemia characterized by ail the following Except: {kasr}

a. Increased fragility of RBCs. b. grey line of gums.

c. Punctate basophilia. d. Reticulocytosis.

46-For cases of chronic lead toxicity, X-ray bones will show: {kasr}

a. Decreased bone density.

b. Lead lines at distal ends of growing bones

c. Multiple callus formation indicating repeated episodes of toxicity.

d Starts to appear after 10 years of exposure

47-The following metal will show a stage of apparent recovery after acute toxicity: {kasr}

a. Arsenic. b. Lead. c. Iron. d. Mercury.

48-Latent or quiescent stage: {kasr}

a. Is a stage of apparent recovery within 6-24 hrs post ingestion.

b. Occurs in cases of acute iron toxicity.

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c. Both a &b

d. Is due to rapid excretion of iron through GIT.

49-All of the following but one are characters of heavy metals toxicity: {kasr}

a. Toxicity can occur through oral ingestion only.

b. They bind with SH group of enzymes.

c. Gastric lavage can be done safely.

d. They can be detected by atomic absorption spectrometer.

50-Calcium deficiency can aggravate toxicity by: {kasr}

a. Mercury. b. Lead c. Iron. d. Cadmium.

51-Metals combine with tissue enzymes and inhibiting them. {kasr}

a. true b. false

52-Hair examination is helpful for suggesting duration of exposure to metals. {kasr}

a. true b. false

53-Putrefaction is rapid in acute iron toxicity. {kasr}

a. true b. false

54-Desferioxamine is a universal antidote for all types of metal toxicity. {kasr}

a. true b. false

55- The following poison produces punctate basophilia:

a. Lead. b. Cocaine. c. HCN. d. Paracetamol. e. Opium.

56-Wrist drop in chronic lead poisoning is due to paralysis of

A. Motor nerves only. B. motor and sensory nerves.

C. Trigeminal nerve D. Sensory nerves only

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57-……. Helps deposition of lead in bone

A. Alkalis B. Vit deficiency C. EDTA D. Ca deficiency

58-In acute iron toxicity, metabolic acidosis occur in ……. Stage

A. First. B. Second. C. Third. D. Fourth.

59-Male patient, was received at Sohag PCC with a history of ingestion 90 tablets from unknown drug 10 hours ago, he presented with vomiting blood, melena, and diagnosed as acute iron toxicity, after 4 days he died. The cause of death was

a)Early shock b)Liver failure c)Acid base disturbance d)Pulmonary embolism

60-child patient took 20 tablets of sugar coated drug and treated by desferrioxamine, he was diagnosed as

a)Mercury poisoning b)Lead poisoning c)Arsenic poisoning d)Iron poisoning

61- Peripheral neuropathy in chronic lead toxicity is:

a. Pure motor b. Pure sensory

c. Mixed but more sensory d. Mixed but more motor

62-Cause of death of iron toxicity in stage 1……., stage 4……:

a. Cardiac shock, hepatic failure b. Neurogenic shock, renal or hepatic failure

c. Renal failure, lung fibrosis d. Hypovolemic shock, renal or hepatic failure

63-Symptoms subside as iron is taken by reticulo- endotherlial system its occur in stage..:

a. 1 b. 4 c. 3 d. 2

64-Which chelation therapy of iron toxcitiy and when stop it use:

a. EDTA, when vine rose color in urine disappears

b. DMSA, when vine rose color in urine appears

c. Dicobaltedtede, when vine rose color in urine appears

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d. Desferrioxamine, when vine rose color in urine disappears

65-Cancrum Oris can be seen in chronic toxicity by:

a) Mercury. b) Iron. c) Lead. d) Arsenic.

66-In iron poisoning, coma, shock, coagulopathy and hepatic failure occur in:

a) Stage I. b) Stage II. c) Stage III d) Stage IV.

67-Blue line on gums in a battery worker is mostly due to presence of:

a. Lead oxide. b. Lead acetate. c. Lead sulphide d. Lead carbonate.

68-Peripheral neuropathy in chronic lead poisoning is:

a. Mainly motor. b. Purely sensory. c. Mainly sensory. d. Purely motor.

69-All of the followings are true regarding irritants Except

a. Their toxic manifestations appear immediately. b. All have double actions.

c. Gastric lavage can be safely performed.

d. Chelators are their physiological antidotes.

e. Most of them are not affected by putrefaction.

70-The followings are true values of lead Except:

a. It is up to 10 microgram/dl in blood. b. It is up to 0.05 mg/litre in urine.

c. It is up to 5 mg/day in stool. d. MAC is 0.2 mg of lead dust/m in air.

e. Fatal dose is 10g lead acetate.

71-All of the followings are investigations of lead poisoning Except:

a. Blood lead level. b. Urinary lead level. c. Blood picture.

d. Desferrioxamine provocative test. e. X-ray of long bones.

72-Mercurialism is characterized by all of the followings Except:

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a. Burning metallic taste. b. Kinetic tremors. c. Erethism.

d. Metabolic acidosis. e. Fulminate itching.

73-the following statements are true regarding iron toxicity Except:

a. Toxicity is due to presence of free iron in the circulation.

b. The first stage starts after a latent period of 24 hours.

c. Second stage is characterized by disappearance of manifestations.

d. The third stage is characterized by circulatory failure.

e. Intestinal obstruction can occur after 2-5 weeks.

74-Metallic poisons share the following character:

a. Gastric lavage can be safely done. b. Have a local action.

c. Have a selective remote action d. Some of them are re-excreted in G.I.T.

e. All of the above.

75-Lead colic is not characterized by being:

a. Relieved by abdominal pressure. b. Associated with pallor.

c. Associated with fever. d. Relieved by 1.V. Ca injection.

e. Associated with constipation.

76-Encephalopathy occur most rapidly in poisoning with the following lead compound:

a. Lead carbonate. b. Lead pentaoxide. c. Lead acetate.

d. Tetra-ethyl lead. e. Lead arsenate.

77-Circumoral pallor is specially present in cases of

a. Acute lead poisoning. b. Chronic lead poisoning. c. Atropine poisoning

d. Mercurialism. e. Opium dependence.

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78-In a 17 year old patient working in a factory of batteries, pale with constipation and black stool, an x-ray on long bones may show:

a. Osteoporosis. b. Widening of medullary cavity.

c. transverse lines present at ends. d. Calcification of periosteum.

e. All of the above.

79-Sterility, abortion, chronic nephritis, optic atrophy & anemia complictions of chronic poisoning by:

a. Mercury. b. Lead c. Phosphorus. d. Arsenic e. None of the above.

80-Black stool and blue line on gums due to presence of:

a. Lead oxide b. Tetra-ethyl lead. c. Lead sulphide.

d. Lead carbonate. e. Lead acetate.

81-In acute lead poisoning, gastric lavage be done by the following

a. precipitating antidote: Tannin. b. Magnesium sulphate. c. Egg albumin

d. Copper sulphate. e. Calcium hydroxide.

82-Dysentery is a feature of poisoning by the salts of:

a. Mercury. b. Arsenic. c. Iron.

d. Lead e. All of the above

83-Kinetic tremors manifestations of chronic poisoning by:

a. Iron b. Lead d. Mercury. c. All of the above.

84-Mercurialism is characterized by the following Except:

a. Excessive salivation. b. Grey line develops on the gums. c. Static tremors.

d. Nephrotic syndrome. e. Erethism.

85-The following is a haemopoietic disorder in case of plumbism:

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a. Formation of methemoglobin. b. Leukemia.

c. Microcytic hypochromic anemia. d. Leucocytosis. e. c & d.

86-A renal finding in case of plumbism is:

a. Chronic interstitial nephritis. b. Nephrotic syndrome.

c. Fatty degeneration of renal tubular cells. d. All of the above.

e. None of the above.

87-A case of plumbism may show the following Except:

a. Diminution of vision. b. Tingling & numbness. c. Wrist drop.

d. Encephalopathy. e. Constipation.

88-Mobilization of lead from bone is helped by the following agents Except:

a. Ammonium chloride. b. Potassium iodides.

c. Na citrate. d. Parathyroid hormone.

89-The following poison produces circumoral pallor:

a. Lead b. Atropine c. HCN d. Paracetamol. e. Opium.

90-The following poison produces punctate basophilia:

a. Lead. b. Cocaine. c. HCN. d. Paracetamol. e. Opium.

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1. A	2. A	3. A	4. A	5. B
6. B	7. D	8. B	9. A	10.C
11.B	12.C	13.C	14.B	15.B
16.A	17.C	18.B	19.E	20.C
21.A	22.C	23.A	24.B	25.B
26.D	27.B	28.D	29.B	30.C
31.D	32.D	33.B	34.B	35.D
36.D	37.A	38.D	39.C	40.C
41.D	42.D	43.C	44.B	45.B
46.B	47.C	48.C	49.A	50.B
51.A	52.A	53.B	54.B	55.A
56.A	57.A	58.C	59.B	60.D
61.A	62.D	63.D	64.D	65.A
66.C	67.C	68.A	69.A	70.C
71.D	72.D	73.B	74.E	75.C
76.D	77.B	78.C	79.B	80.C
81.B	82.A	83.D	84.C	85.C
86.A	87.B	88.C	89.A	90.A

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Plant poisons

1- The pure antagonist of morphine is: {Mid2021}{Mid2025}{kasr}

a. Naloxone b. Nalorphine c. Atropine d. Digibind

2- Coke burns are localized necrotic ulcers produced by: {Formative2023}{Mid2021}{Final2025}{kasr}

a. Dry burn b. Hot liquids c. Subcutaneous cocaine injection d. Electrocution

3-Methods of elimination of barbiturate include: {Formative2025}{Final2023}

a) Hemofilteration b) Hemodialysis c) Alkalization of urine d) All Correct

4-Atropine present with the following except {Final2025}

A-Hypothermia B-Flushed face

5-Which of the following is the cutaneous manifestations of barbiturates coma? {Final2025}

A-Echymosis B-Petechial hemorrhage

C-Bullae surrounded with hyperemia D-Dry skin

6- A 7-years old boy ingested unknown seed then developed dilated pupils, flushing, tachycardia these seeds mostly? {Mid2024}

A. Datura B. Papaver somniferous C. Strychnous nux vomica

7- coma, respiratory depression, pin point pupil is Triade of {Mid2024}

A. Opium

8-Classic triad of opiate over dose is: {Qena}

a. Coma, mydriasis & respiratory depression b. Coma, miosis & hyperthermia

c. Coma, miosis & respiratory depression d. Coma, mydriasis & hypothermia

9-Manazool is composed of {Mid2023}

a) datura, hashish, nutmeg, sugar b) datura, hashish, nutmeg, honey

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c) atropine, hashish, cocaine, sugar

10- Cause of death in cocaine {Final2023}

A- I.C hemorrhage B- Renal failure C- hypothermia D- hypotension

11- Sense of well-being followed by fear of death in {Mid2024}

A. Acute cocaine toxicity B. Acute atropine toxicity

C. Acute cannabis toxicity D. Acute morphine toxicity

12- Cannabis is {Final2023}

A- hallucinogenic b) CNS stimulant c) CNS depressant

13-disorienation of time and space occurs in the poisoning of {Mid2023}

a) aspirin b) atropine c) THC d) none of the above

14- Acute cocaine toxicity causes all of the following except {Mid2024}

A. Coronary vasoconstriction leading to myocardial infarction

B. Hypertension leading to cerebral hge C. Seizures D. Hypothermia

15- A 25 years old man came to hospital with hypotension, bradycardia, pin point pupil and there is multiple injection marks along veins of upper limb ? {Mid2024}

A. Atropine toxicity B. Morphine toxicity C. Alcohol toxcicty D. Hashish toxcicty

16-Morphine toxicity is characterized by which of the following? {Mid2023}

a) Shock b) Respiratory depression c) Mydriasis d) All the above

17-in Strychnine poisoning cause of death is {Mid2025}

A-Hypertension B-Lactic acidosis C-asphyxia D-vomiting

18-Which one of the following is used as opiate antidote? {Merit}

a. N-acetylcysteine b. Pilocarbine c. Naloxone d. Oximes

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19-Which one of the following is used as atropine antidote? {Merit}

a. Naloxone b. Oximes c. N-acetylcysteine d. Pilocarbine

20- All are active principles papaver somniferum plant EXCEPT: {Final2021}{kasr}

a. Thebaine b. Papaverine c. Diacetyl morphine d. Codeine

21- Atropine toxicity is not accompanied with: {Final2021}

a. Tachycardia b. Dry air passages c. Hypothermia. d. Hallucinations

22- The end point of atropine use In treatment of poisoning is: {Final2021}

a. The pupil dilates. b. Heart rate increase organophosphorus.

c. Bronchospasm relieved d. Vomiting stopped.

23-Crushed strychnus nux vomica seed isnot toxic {Formative2025}

a) True b) False

24-All are active principles of papaversomniferum plant except: {Formative2022}

a. Thebaine b. Papaverine c. Diacetyl morphine d. Codeine

25-The active principles of thorn apple are extracted from: {Formative2022}

a. Seeds b. Leaves c. Roots d. All of the above

26-Opium intoxication produces all of the followings except: {Formative2022}

a) Hyperthermia b) Slow pulse c) Hypothermia d) Pin point pupils

27-Cutaneous blisters that are typically found on the dependent part are found in: {Formative2022}

a) hypnotics b) Co c) prolonged coma d) All of the above

28- "Coke burn" is produced due to {Final2022}{Mid2022}

A- Cigarette burn. B- Flame injury.

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C- Electric born. D- Subcutaneous injection of cocaine

29- All of the following are characters of barbiturates coma EXCEPT {Final2022}{kasr}

A- Rapid shallow respiration B- Hypothermia C- Hypotension D- Diminished reflexes

30-All of the following are characters of barbiturates coma except {Formative2023}

a. Respiratory depression b. Hypothermia c. Hypertension d. Confusion

31-All are manifestations of beta blockers overdose except {Formative2023}

a. Hypoglycemia b. Hypokalemia c. Bradycardia d. Hypotension

32-Group of friends like to be together in every thing, all of them are addict to opioids. In a party they take a toxic dose of opioid, but by different method of administration. What is the root of administration with very rapid toxic symptom?

a. Intramuscular b. Oral c. Intravenous d. Subcutaneous

33-atropine fever is due to: {قديم}{Qena}

a-stimulation of heat regulating center b-depressed sweating

c-increased metabolic rate d-all of the above

34-physical dependence occur in which type of opioid receptor: {مهم}

a. sigma b. Kappa c. MU d. delta

35-Which that characterized of C/P of hashish toxicity: {مهم}

a. Nasal septal perforation b. Seizures, status epilepticus

c. Disorientation to time and space d. Hyperthermia

36- Disorientation to time and space is seen in poisoning with: {Qena}

a. alcohol b. atropine c. opium d. hashish

37- Prolonged time and space perception is due to {Qena}

a) Atropine toxicity b) Cannabis toxicity

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c) Morphine toxicity d) Strychnine toxicity

38-Barbiturate coma is characterized by {Qena}

a) Flushed skin with dilated pupils.

b) Deep profound coma with cyanosis and constricted pupils

c) Flushed skin with dilated reactive pupils.

d) Deep profound coma with cyanosis and dilated reactive pupils

39- all of the following cause fixed dilated pupil except {Qena}

A. Nicotine B. Cocaine C. Opiates D. Atropine

40-Mydriasis, tachycardia, urinary retention are toxidromes for: {Qena}

A) Hyoscine B) Digitalis C) Salicylates D) All of the above

41- The classic morphines triad: {قسم}

a. Coma,cyanosis,pinpoint pupil b. Convulsions,cyanosis,pinpoint pupil

c. Coma,cyanosis,dilated reactive pupil d. Tachyarrythmia,cyanosis,pinpoint pupil

42- The first line in treatment of morphine poisoning: {قسم}

a. Antidote naloxone b. Gastric lavage

c. Blood exchange or dialysis d. Emergency measures ABC

43- Opium has a characteristic odor but morphine hasn't {قديم}

A- true B- False

44- The best antidote for Morphine poisoning is {قديم}{Qena}

A- Nalorphine B- levalorphane C- Naloxone D- atropine

45- Atropine is an antidote for all of these poisons except: {قسم}

a. Organophosphorus insecticides b. Aconitine and morphine

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c. Digitalis d. Cocaine

46- Acute cocaine toxicity causes all of the following except: {قسم}

a. Euphoria and seizures b. Myocardial infarction c. Pinpoint pupil d. Hyperthermia

47-A perforated nasal septum is an indication for addiction to: {Qena}

A) Hashish B) Amphetamine. C) Cocaine D) Heroin

48-Pin point pupil occurs in those poisons, except: {kasr}

a. Opioids b. Carbamate c. Phenol. d. Anticholinergic drugs.

49-Active principles of "thorn apple" are: {kasr}

a. Atropine, and hyoscine b. Codeine and morphine

c. Digoxin and digitonin d. None of the above

50-The Active principles of "thorn apple" are extracted from: {kasr}

a. Seeds b. Roots c. Leaves d. All of the above

51-Alkaloids have: {kasr}

a. Remote action only b. Local action only

c. Local and remote action only d. All of the above

52-Alkaloids' local antidotes are all except: {kasr}

a. Charcoal b. Hydrogen peroxide c. Naloxone d. NaHCO3

53-All are Uses of Atropine except: {kasr}

a. Antispasmodic b. Diuretic c. Mydriatic d. Bronchodilator

54-Which of the following agents is not considered a central nervous system depressant? {kasr}

a. Barbiturates b. Opiates c. Ethanol d. Cocaine

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55-Atropinism includes all of the following except: {kasr}

a. Constipation and urine retention b. Atropine fever

c. Talk and visual hallucination d. Dysphagia due hoarseness of voice

56-Atropine toxicity is never accompanied with: {kasr}

a. Tachycardia b. Dry air passages c. Hypothermia d. Euphoria

57-Atropine toxicity is characterized by: : {kasr}

a. Drunken gait b. Dilated fixed pupil c. Central asphyxia d. All of the above

58-All of the followings are the clinical picture of morphine toxicity except : {kasr}

a. Respiratory depression b. Smell of meconic acid

c. Euphoria or dysphoria d. Hypothermia

59-Opium toxicity may show all of the followings, except: {kasr}

a. Hypothermia b. Miosis & fixed pupil

c. Diminished bowel sounds d. None of the above

60-The central effect of atropine is not responsible for: {kasr}

a. Occupational Delerium b. Drunken gait

c. Hyperthermia d. None of the above

61-Comatose patient with morphine toxicity never shows: {kasr}

a. Diminished bowel sounds b. Dyspnea c. Hypertension d. Hypothermia

62-All may cause the triad of classic opioids toxidrome, except: {kasr}

a. Carbolic acid b. Morphine c. Hyoscine d. Organophosphorous

63-All are Pure Antagonist Antidotes of morphine, except: {kasr}

a. Nalmefene b. Naloxone c. Nalorphine d. Naltrexone

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64-Hashish is extracted from: {kasr}

a. The flower top. b. Dried leaves.

c. Resin extracted from the flowers. d. Mixture of flower & leaves.

65-All of the followings are routes of administration of hashish, except: {kasr}

a. Smoking. b. Ingestion. c. Inhalation. d. Injection.

66-Chronic cannabis toxicity leads to: {kasr}

a. Sterility. b. Increased testosterone level.

c. Increased sperm count. d. All of the above.

67-All the followings are routes of administration of cocaine, except {kasr}

a. Smoking. b. Ingestion. c. Inhalation. d. Injection.

68-Cocaine has the following mechanism of action: {kasr}

a. Sympathomimetic effect. b. CNS stimulation.

c. Local anesthetic effect. d. All of the above.

69-Cardiovascular manifestation of cocaine toxicity include: {kasr}

a. hypotension b. bradycardia

c. coronary artery spasm d. All of the above

70-The major cause of death is stimulatory stage of cocaine toxicity is {kasr}

a. Hypertension b. Circulatory collapse c. Coke burn d. Hyperthermia

71-Increased appetite to sugar is common with the intake of: {kasr}

a. Opium b. Atropine c. Cannabis d. Cocaine

72-Dilated pupils occur in the following poisoning, except: {kasr}

a. Opium. b. Amphetamine. c. Cyanides. d. Atropine.

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73-Addicts prefer heroin to morphine because: {kasr}

a. It is less constipating. b. Its action is faster.

c. It is more euphoric. d. All of the above.

74-Heroin is: {kasr}

a. Di-methyl morphine b. Di-acetyl morphine.

c. Tri-methyl morphine. d. Tetra-acetyl morphine.

75-All of the followings can cause hypothermia except: {kasr}

a. Opium b. Barbiturates. c. Ethanol d. Cocaine

76-A heroin overdose is characterized by all of the following symptoms except: {kasr}

a. Hypotension b. Diminished reflexes c. Mydriasis d. Coma

77-All the following intoxications. associated with miosis except:{kasr}

a. Neostigmine (Prostigmine) b. Morphine derivatives

c. Organic phosphate esters d. Cocaine

78-Atropine is used for treatment of the following drugs poisoning, except: {kasr}

a. Morphine b. Digitalis

c. Organophosphorus compounds d. Organochlorine insecticide

79-Naloxone [Narcan] is not used in treatment of: {kasr}

a. Clonidine toxicity b. Opioids addiction

c. Coma of unknown etiology d. Non of the above

80-Naloxone [Narcan]& Naltrexone [Trexan] both are Pure Antagonist Antidote with no difference{kasr}

a. true b. false

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81-Naloxone [Narcan] is used in the treatment of opioid addiction not in its diagnosis {kasr}

a. true b. false

82-Hyoscine is used as "Truth serum" & in mania {kasr}

a. true b. false

83-Pilocarpine is the best antidote of Hyoscine {kasr}

a. true b. false

84-Atropine. Hyoscyamine and Hyoscine have the same actions {kasr}

a. true b. false

85-Mechanism of digitalis action in therapeutic dose is the same as in toxic dose {kasr}

a. true b. false

86-Insulin in 5% glucose is used in the treatment of acute digitalis toxicity not in chronic cases {kasr}

a. true b. false

87-Sexual act is lengthened with hashish intake. {kasr}

a. true b. false

88-Car accident is common cause of death in cannabis toxicity. {kasr}

a. true b. false

89-Perforation of nasal septum can occur with cocaine addiction. {kasr}

a. true b. false

90-Cocainism causes perforated nasal septum. . {kasr}

a. true b. false

91-All of the following is C/P of cocaine toxicity EXCEPT

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a. Seizures, status epilepticus b. Hypertension, Bradycardia, hyperthermia

c. Renal and myocardial vasospasm d. Nasal septal perforation

92-antidote of morphine toxicity is:

a. atropine b. codeine c. naloxone, nalorphine d. oxime

93-antidote of atropine toxicity in severe cases:

a.oxime b.physostigmine c. naloxone. d. None

94- The following poison produces flushed skin:

a. Lead. b. Cocaine. c. Atropine. d. Opium.

95-all of the following is C/P of atropine EXCEPT:

a.pin point and fixed puplis b.dryness in all secretion

c. hallucination (visual, auditory) d. occupational delirium

e. tachycardia, hypotension

96-Morphine causes death mainly by:

A) Respiratory muscle spasm B) Paralysis of respiratory center

C) Cardiac rupture D) Kidney failure

97-Strychnine poisoning leads to death due to:

A) Paralysis of respiratory center B) Spasm of respiratory muscles

C) Liver failure D) Hypovolemia

98-In atropine CNS toxicity the antidote of choices:

a. Pyridostigmine. b. Pilocarpine. c. Neostigmine. d. Physostigmine.

99-As regard Physostigmine, The following side effect is common:

a. Hypertension. b. Hallucinations. c. Tachycardia. d. Asystole.

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100-The classic triad of acute morphine toxicity is:

a) Coma, hypotension and respiratory depression.

b) Coma, abolished reflexes and pin point pupils.

c) Coma, hypotension and cyanosis.

d) Coma, pin point pupils and respiratory depression

101-A characteristic urinary manifestation of atropine toxicity is:

a) Incontinence. b) Retention. c) Dysuria d) Oliguria

102-The cardio-stimulatory effects of cocaine are due to:

a. Blocking vagal tone. b. Increasing vagal tone.

c. Increasing reuptake of catecholamines. d. Blocking reuptake of catecholamines.

103-Coke burns are diagnostic of:

a. Opiate toxicity b. Alcohol toxicity. c. Cocaine toxicity. d. Cannabis toxicity.

104-A case of coma with pin point pupils, hypothermia, bradypnea (5/min), constipation and hyporeflexia, is mostly due to:

a. Opium toxicity. b. Organophosphate toxicity.

c. Barbiturate toxicity. d. Ethanol toxicity.

105-A person was brought by police from the railway platform. He is talking irrelevant. He is having dry mouth with hot skin, dilated pupils, staggering gait and slurred speech. The most probable diagnosis is:"

a. Alcohol intoxication b. Carbamates poisoning

c. Organophosphorus poisoning d. Datura poisoning

106-The most obvious clinical feature in cannabis overdose is:

a. Tachyarrhythmia. b. Disorientation in time &space

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c. Hypertension. d. Tinnitus.

107-All the followings are alkaloids Except

a-Atropine b- Cocaine. c-Digitalin.

d- Morphine. e- Strychnine

108-Opium contains all the followings Except:

a-Morphine. b- Codeine c- Thebaine d-Heroin e- Papaverine

109-The followings are opioid receptors Except:

a. Mu b. Zeta. c. Kappa. d. Sigma e-Delta.

110-Morphine inhibits all the followings Except:

a- Cerebral cortex b- Respiratory center. c. Cough center

d-CRTZ e. VMC.

111-In morphine coma, the following may be found Except:

a- Abolished reflexes. b- Dilated pupils c- Slow respiration.

d-Hypothermia. e-Cyanosis.

112-The following plants contain atropine except

a- Datura alba b- Atropa belladonna. c- Colocynth plant.

d- Datura niger. e- Hyoscyanus muticus.

113-Therapeutic uses of atropine include the following Except:

a- Preanaesthetic medication. b- In organophosphate poisoning.

e- In digitalis poisoning. d-In DNP poisoning. e-In aconitine poisoning.

114-In cocaine toxicity, all of the following are true Except:

a. There is initial euphoria and excitement.

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b. Tachycardia may be fatal ventricular fibrillation.

c. Temperature may be decreased.

d. Renal failure may occur.

e. There may be chest pain without ECG changes.

115-All the following are lines of treatment of cocaine poisoning Except:

a. K-permenganate gastric lavage. b. I.V. diazepam. c. Application of ice bags.

d. B-blockers. e. Respiratory stimulants.

116-Dysphagia is a feature of poisoning by:

a. Morphine. b. Cocaine. c. Hashish. d. Atropine. e. Digitalis.

117-The followings are typical signs of atropine poisoning Except:

a. Delirium. b. Dysphagia. c. Hypothermia.

d. Mydriasis. e. Tachycardia.

118-Which one of the following poisons produces fever, dilated pupil & staggering gait:

a. Cocaine. b. Opium. c. Ethanol. d. Hashish. e. Atropine.

119-Tachycardia in atropine poisoning is due to its effect as:

a. Vasomotor stimulant. b. Cardiac stimulant. c. Anticholinesterase.

d. Anticholinergic. e. Respiratory stimulant.

120-A patient is flushed, talkative, staggering & feverish in poisoning by:

a. Cocaine. b. Ethanol. c. Atropine. d. Hyoscine e. opium

121-Hypothermia occurs in poisoning by:

a. Opium. b. Hashish. c. Ethanol.

d. All of the above. e. None of the above.

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122-In acute opium poisoning the pulse is:

a. Slow. b. Rapid. c. Irregular.

d. Normal. e. None of the above.

123-Gastric lavage in case of morphine poisoning is followed by giving:

a. Lime water. b. Magnesium sulphate. c. Milk.

d. Egg white. e. Vinegar.

124-The active principle of papaver somniferum is present in:

a. Stem b. Flower c. Leaves

d. Capsules e. All of the above.

125-Delta 9 tetrahydrocannabinol is considered as:

a. Narcotic analgesic. b. Sedative c. Hallucinogen.

d. Tranquilizer. e. All of the above

126-The active principle of cannabis sativa is mainly present in:

a. Male flowering tops. b. Female flowering tops. c. Male juice.

d. Female juice. e. All of the above

127-Perception of time & space is prolonged due to the intake of:

a. Cocaine. b. Hashish. c. Morphine. d. atropine

128-The heart rate is rapid after ingestion of:

a. Amphetamine. b. Atropine. c. Hyoscyamine.

d. Cocaine. e. All of the above.

129-The action of cocaine on the CNS is:

a. Depression only. b. Stimulation only. c. Stimulation then depression.

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d. Depression then stimulation. e. Tranquillization

130-Convulsions are usually absent in case of poisoning by:

a. Cocaine. b. Opium c. Atropine d. Oxalic acid.

131-The best antidote for acute opium poisoning is:

a. Physostigmine. b. Atropine sulphate. c. Calcium.

d. Naloxone. e. Pilocarbine.

132-The best antidote for severe atropine poisoning is:

a. Physostigmine. c. Calcium. b. Atropine sulphate

d. Naloxone e. Pilocarbine.

133-The following poison produces flushed skin:

a. Lead. b. Cocaine. c. Atropine. d. Paracetamol e. Opium.

1. A	2. C	3. D	4. A	5. C
6. A	7. A	8. C	9. B	10.A
11.C	12.A	13.C	14.D	15.B
16.B	17.C	18.C	19.D	20.C
21.C	22.C	23.B	24.C	25.D
26.A	27.D	28.D	29.A	30.C
31.B	32.C	33.D	34.C	35.C
36.D	37.B	38.D	39.C	40.A
41.A	42.D	43.A	44.C	45.D
46.C	47.C	48.D	49.A	50.D
51.A	52.C	53.B	54.D	55.C
56.C	57.D	58.B	59.D	60.C
61.C	62.C	63.C	64.A	65.D
66.A	67.B	68.D	69.C	70.D
71.C	72.A	73.D	74.B	75.D
76.C	77.D	78.D	79.B	80.B
81.B	82.A	83.B	84.B	85.B
86.A	87.B	88.A	89.A	90.A

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91.B		92.C		93.B		94.C		95.A	
96.B		97.B		98.D		99.D		100.	D
101.	B	102.	D	103.	C	104.	A	105.	D
106.	B	107.	C	108.	D	109.	B	110.	D
111.	B	112.	C	113.	D	114.	C	115.	D
116.	D	117.	C	118.	E	119.	D	120.	C
121.	D	122.	A	123.	B	124.	D	125.	C
126.	B	127.	B	128.	E	129.	C	130.	B
131.	D	132.	A	133.	C	134.		135.	

Toxicity of some common Drugs

1- We can use activated charcoal in {Mid2025}

A- lithium B- aspirin C- Strychnine D- benzene

2-In paracetamol over dose, the toxic metabolite NAPQI is normally detoxified by the following? {Final2025}

A-Glutathione B-P450 enzyme

3- Which of the following is antidote of acetaminophen: {Final2023}{Formative2022}

A- toxogonin B- N-acetyl cysteine C- naloxone D- atropine

4-Antidote of paracetamol toxicity is {Formative2025}

a) N-acetyl cysteine b) Vit K c) Naloxone d) Cognetine

5-Aspirin toxicity produce the following manifestations except: {Formative2025}{Final2023}

a) Extrapyramidal b)Ecchymosis c)Vomiting d)Tinnitus

6- To enhance elimination of salicylates the most suitable method before deciding hemodialysis is: {Mid2021}

a. Forced acidic diuresis. b. Forced alkaline diuresis. c. Activated charcoal.

d. Oral sodium bicarbonate to make easy complex with salicylates can be excreted.

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7- All types of arrhythmias can be occurred with digoxin toxicity EXCEPT: {Final2021}

a. First type heart block b. Bradycardia

c. SVT with rapid conduction, d. SVT with slow conduction

8- Flumazenil ? {Mid2022}

A- Is used in combined toxicity of benzodiazepines and antidepressants.

B- Has long duration

C- Induces cardiac conduction disturbances in benzodiazepine dependence.

D- None of above

9- Phase 1 of acetaminophen toxicity doesn't include {Mid2024}

A. Vomiting B. Malaise C. Pallor D. Jaundice

10-Antidote of B blockers? {Mid2024}{Final2025}

A-Glucagon B-Digibind C-N acety cysteine D-Atropine

11- Most dangerous form of Salicylates {Mid2024}

A. Acetyl salicylic acid B. Salicylic acid tablets

C. Methyl Salicylate (oil of wintergreen) D. Salicylic acid capsules

12- 35Y female with history of suicidal attempt by 20 tablet acetaminophens, the most important line of treatment is: {Final2023}

A- atropine B- obidoxime C- N-acetyl cysteine D- resuscitation

13- A 35-year-old female patient came to the hospital comatosed and cyanosed with history of suicidal attempt by malathion,20 tablets of acetaminophen 500 mg and 30 tablets of folic acid. The most important line of treatment in this situation? {Final2022}

A- Atropine B- Obidoxim C- N-acetylcystein D- Resuscitation

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14- A 18-year-old comatose female was admitted to ER after committing suicide through ingestion of some unknown tablets. Examination showed pulse rate was 55/min, blood pressure was 85/50, bronchospasm, hyperkalemia and hypoglycemia. The Condition deteriorated to repeated convulsions and coma. The most possible diagnosis? {Final2022}

A- Acute beta-blocker toxicity B- Acute paracetamol toxicity

C- Acute amphetamine toxicity D- Acute cocaine toxicity

15- All poisoning are causes of hypothermia except: {قسم}

a. Carbon monoxide b. Opiate c. Salicylate d. Ethanol

16- Salicylate poisoning cause bleeding tendency due to except {قديم}

A- Decrease prothrombine formation B- Decrease platelet aggregation

C- Increase capillary fragility D- Decrease vitamin K absorption

17- Acute severe salicylate overdose can cause death through the following: {Qena}

a. Metabolic acidosis. b. Liver cell failure.

c. Bleeding per rectum. d. Deep venous thrombosis.

18-Manifestation of acute digitalis toxicity include all the following except: {Qena}

a. hyperkalemia. b. hypokalemia. c. heart block. d. protracted vomiting

19- After complete management of digitalis toxicity the following should be monitored; {Qena}

a) PH. b) K level. c) Urine output. d) Creatinine level

20- Digitalis glycosides produce their toxic action by inhibiting {قديم}

A- Monoamine oxidase B- Na-K-ATPase C- Carbonic anhydrase D- Choline esterase

21-Yellow vision perception ocurrs in toxicity with: {Qena}

a- Salicylates b- Nicotine c-Alcohol d-digitalis

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22- Over dose of aspirin causes pyrexia. {Qena}

a. True b. False

23-Start of Elevation of serum AST & ALT in paracetamol overdose occur in: {Qena}

a.Phase 1 b.Phase II c.Phase III d.Phase IV

24-All can happen with salicylate overdose except: {Qena}

a. Hyperthermia b. Nerve paraysis. c. Nephrotoxicity d. Increased prothrombin time

25-Acute salicylate poisoning with signs of renal failure can be treated by all of the following except: {قديم}{Qena}

a. Heamodialysis b. Forced alkaline diuresis c. Vit. K d. Gastric lavage with water

26- Hepatic failure in paracetamol overdose is due to {قديم}{Qena}

A- Metabolic acidosis B- Direct action of the drug

C- The action of its metabolite D- Vomiting and dehydration

27-The main line of treatment in paracetamol overdose is: {Qena}

a-Hemodialysis b-Antidote therapy c-Hemoperfusion d-Fluid replacement

28-Aspirin increases all the following, except: {kasr}

a. Bleeding time b. Glycolysis. c. K serum levels. d. Prothrombin time.

29-Aspirin causes dehydration due to all the following. except: {kasr}

a. Increased metabolic rate. b. Increased renal blood flow.

c. Sweating. d. Hyperventilation.

30-Aspirin causes: {kasr}

a. Metabolic alkalosis followed by respiratory acidosis.

b. Respiratory acidosis followed by metabolic alkalosis.

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c. Metabolic acidosis followed by respiratory alkalosis.

d. Respiratory alkalosis followed by metabolic acidosis.

31-Aspirin causes metabolic acidosis by all the following mechanisms, except: {kasr}

a. Stimulation of Kreb's cycle. b. Stimulation of lipid metabolism.

c. Inhibition of amino acid metabolism. d. Uncoupling of oxidative phosphorylation.

32-Aspirin causes metabolic acidosis by all the following mechanisms, except: {kasr}

a. Accumulation of amino acids. b. Accumulation pyruvic acid.

c. Accumulation gamma-amino butyric acid d. Depletion of HCO3

33-Uncoupling of oxidative phosphorylation causes the: {kasr}

a. Increased glycogenesis. b. Increased glycolysis

c. Increased ATP production. d. Inhibition of gluconeogenesis.

34-Toxic dose of paracetamol causes depletion of: {kasr}

a. Glucuronic acid. b. Glutathione. c. Sulfate d. Cytochrome P450.

35-Phase I of paracetamol overdose comprises all the following, except: {kasr}

a. Nausea & vomiting. b. Sweating. c. Jaundice. d. Malaise.

36-Elevated serum AST & ALT in paracetamol overdose occur in: {kasr}

a. Phase 1 b. Phase II c. Phase III d. Phase IV

37-Barbiturates are used as all the following, except: {kasr}

a. Analgesic. b. Hypnotic. c. Sedative. d. Anesthetic.

38-Barbiturates cause CNS depression by making GABA dependent chloride channel: {kasr}

a. Inactive. b. Open Longer. c. Open normal d. Open shorter.

39-Reflexes in barbiturates toxicity are: {kasr}

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a. Exaggerated. b. Normal. c. Diminished. d. Absent.

40-Hypotension in barbiturates overdose is due to all the following mechanisms except: {kasr}

a. Depression of vasomotor center b. Depression of myocardium.

c. Decreased Blood volume.

d. Compromised vascular tone (vasoplegic effect)

41-One of the delayed causes of death in barbiturates toxicity is: {kasr}

a. Central asphyxia. b. Cardiovascular collapse c. Hypothermia. d. Pneumonia

42-The following statements are true as regards BZ. except: {kasr}

a. They have high therapeutic to toxic indices.

b. They are the first line hypnotics

c. Dependence is not a sequel of their chronic use.

d. They are the most commonly prescribed psychotropic drugs.

43-Benzodiazepines are preferred to barbiturates because: {kasr}

a. Benzodiazepines cause profound coma. b. Barbiturate cause respiration depression

c. Benzodiazepines cause significant hypotension d. Death from isolated BZ is common.

44-Benzodiazepines are the Ist line anoxiolytics because: {kasr}

a. Low grade coma. b. Significant hypotension is uncommon.

c. Death is rare. d. All of the above.

45-Flumazenil: {kasr}

a. Is used in combined toxicity of benzodiazepines & TCAS

b. Antagonistic effect lasts for 10 hours.

c. Induces withdrawal syndromes in chronic BZ us.

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d. None of the above.

46-Blockade of influx of calcium through Ca channels result in all the following. except: {kasr}

a. Vasodilatation. b. Increased cardiac contractility.

c. Depressed nodal conduction. d. All of the above.

47-The cardinal features of Ca antagonist intoxication are: {kasr}

a. Hypertension & tachycardia. b. Hypertension & bradycardia.

c. Hypotension & tachycardia. d. Hypotension & bradycardia.

48-Hypoglycemia is a common finding in all of the following poisoning except: {kasr}

a. B-adrenergic receptor antagonists. b. Ca-channel antagonists.

c. Salicylates. d. Ethanol

49-All are manifestations of beta adrenergic blockers overdose, except: {kasr}

a. Hypoglycemia. b. Hypothermia. c. Bradycardia. d. Hypotension.

50-Glucagon may be effective in treatment of toxicity with: {kasr}

a. Calcium channel antagonists. b. B-adrenergic receptor antagonists.

c. Anti-diabetic drugs. d. All of the above.

51-Patients with chronic theophylline toxicity do not have: {kasr}

a. Hypotension. b. Metabolic acidosis. c. Vomiting. d. Tachycardia.

52-Theophylline is efficiently removed by all the following methods, except: {kasr}

a. Hemodialysis b. Hemoperfusion. C. MDAC. d. None of the above.

53-Hyperkalemia may be caused by each of the following intoxications, except: {kasr}

a. Chronic digoxin poisoning b. Acute digoxin poisoning

c. Beta-adrenergic blockers d. Potassium-sparing diuretics

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54-Benzodiazepines can be used in treatment of all the following cases except: {kasr}

a. Tetanus b. Insomnia c. schizophrenia d. Anxiety

55-All the following statements are true regarding paracetamol, except: {kasr}

a. It has antipyretic and analgesic actions as aspirin

b. Its main hazards are liver damage and renal tubular necrosis

c. It is metabolized almost exclusively in the liver

d. Its toxic dose results in rise of glutathione level.

56-The 1" line of treatment of hypotension and bradycardia of B-blockers toxicity is: {kasr}

a. Atropine b. Glucagon c. Mg So4 d. Lidocaine

57-Following ingestion of toxic dose of acetaminophen, hepatic toxicity begins in: {kasr}

a. Stage I b. Stage II c. Stage III d. Stage IV

58-Which of the following is true about oil of winter green? {kasr}

a. Increased bleeding tendency b. Hyperthermia

c. Urticaria d. All of the above

59-Intoxication of all the following drugs causes coma except: {kasr}

a. Atropine. b. Barbiturates. c. Cocaine. d. Strychnine.

60-Activated charcoal is effective to treat toxicity of: {kasr}

a. Lithium b. Benzene. c. Aspirin d. Strychnine

61-Salicylate poisoning is potentially life-threatening. {kasr}

a. true b. false

62-Accidental salicylate ingestion is common in children. {kasr}

a. true b. false

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63-Bleeding tendency is uncommon in patients with aspirin toxicity. {kasr}

a. true b. false

64-Acid-base imbalance is one of the most common manifestations of salicylate poisoning. {kasr}

a. true b. false

65-Respiratory alkalosis is the final acid-base disturbance in aspirin toxicity. {kasr}

a. true b. false

66-Renal compromise occurs frequently in aspirin toxicity. {kasr}

a. true b. false

67-Hypothermia is one of the manifestations of aspirin toxicity being antipyretic drug. {kasr}

a. true b. false

68-Hepatotoxicity following acute salicylate overdose is very common. {kasr}

a. true b. false

69-Acute lung injury has been reported with severe salicylate intoxication. {kasr}

a. true b. false

70-Cerebral edema never occurs in salicylate poisoning. {kasr}

a. true b. false

71-An overdose of paracetamol leads to overwhelming the hepatic glutathione. {kasr}

a. true b. false

72-Paracetamol is a potent anti-inflammatory drug. {kasr}

a. true b. false

73-Paracetamol is hepatotoxic in overdose. {kasr}

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a. true b. false

74-Tubular necrosis is never caused by paracetamol toxicity. {kasr}

a. true b. false

75-In paracetamol toxicity, activated charcoal is better given in conjunction with the antidote. {kasr}

a. true b. false

76-N-acetyl cysteine (NAC) is the best antidote for paracetamol toxicity. {kasr}

a. true b. false

77-N-acetyl cysteine is only given orally for treatment of paracetamol toxicity. {kasr}

a. true b. false

78-Barbiturates have CNS depressant effect. {kasr}

a. true b. false

79-Barbiturate overdoses may cause hypertension & tachycardia. {kasr}

a. true b. false

80-Barbiturate overdoses may cause pneumonia. {kasr}

a. true b. false

81-Barbiturates are not nephrotoxic drugs; however they may lead to renal failure. {kasr}

a. true b. false

82-Hemodialysis is effective in treatment of barbiturate overdoses. {kasr}

a. true b. false

83-Hemodialysis is more effective than hemoperfusion for treatment of barbiturate toxicity. {kasr}

a. true b. false

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84-Barbiturates may cause rhabdomyolysis. {kasr}

a. true b. false

85-Benzodiazepines are the 1" line anoxiolytics. {kasr}

a. true b. false

86-Barbiturates are safer than BZ. {kasr}

a. true b. false

87-Flumazenil has a much shorter half-life than most oral BZ. {kasr}

a. true b. false

88-Flumazenil is effectively used in management of BZ addiction. {kasr}

a. true b. false

89-Flumazenil is best used in combined toxicity of BZ & other convulsant drugs. {kasr}

a. true b. false

90-Flumazenil has minimal disadvantages and precautions. {kasr}

a. true b. false

91-Death from isolated BZ overdose is rare. {kasr}

a. true b. false

92-Benzodiazepines may cause rhabdomyolysis. {kasr}

a. true b. false

93-Ca channel antagonist intoxication causes hypoglycemia. {kasr}

a. true b. false

94-Ca channel antagonist intoxication causes tachycardia and hypertension. {kasr}

a. true b. false

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95-Calcium chloride is preferable to calcium gluconate in treatment of CCA overdose. {kasr}

a. true b. false

96-Propranolol can cross the blood brain barrier (BBB). {kasr}

a. true b. false

97-Beta adrenergic receptor antagonist intoxication may cause hypoglycemia. {kasr}

a. true b. false

98-Beta adrenergic receptor antagonist intoxication may cause hypokalemia. {kasr}

a. true b. false

99-Beta adrenergic receptor antagonist intoxication may be associated with torsades de pointes. {kasr}

a. true b. false

100-Calcium chloride is not effective in treatment of BARA overdose. {kasr}

a. true b. false

101-Calcium gluconate is effective in treatment of BARA overdose. {kasr}

a. true b. false

102-Theophylline intoxication may cause hypokalemia. {kasr}

a. true b. false

103-Charcoal hemoperfusion is highly effective in theophylline elimination{kasr}

a. true b. false

104-Female admitted to the emergency department with history of ingestion of 100 tablets of aspirin since 18 hours, with investigations the patient had renal failure with serum salicylates level 150 mg/dl. What is the most appropriate method for increase excretion of salicylate in this patient?

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A. Whole bowel irrigation. B. Hemodialysis.

C. Alkalization of urine. D. Acidification of urine

105-Patient admitted to emergency department 6 hours after ingestion of large amount of paracetamol tablets. What is the common complaint of the patient during this period?

A. Nausea, vomiting, pallor, malaise. B. Metabolic acidosis.

C. Hepatic encephalopathy. D. Jaundice.

106-Salicylates is differed from paracetamol is being used as

a. Analgesic b. Antipyretic c. Anti-inflammatory d. Antispasmodic

107-the main metabolite responsible for toxikinetics in paracetamol over dose is

a. glucuronide conjugate b. sulfate conjugate c. NAPQI d. N-acetyl-para-aminophenol

108- Gastric lavage is performed up to 12 hrs post-ingestion in one of the following:

a. Digitalis. b. Paracetamol. c. Salicylates. d. Carbolic acid.

109-The early metabolic disturbance in case of salicylate toxicity is:

a. Respiratory acidosis b. Metabolic alkalosis

c. Metabolic acidosis d. Respiratory alkalosis

110-Which one of the following is a presentation of severe salicylate toxicity

a. Lethargy b. Hyperthermia c. Tinnitus d. Vertigo

111-Paracetamol is selectively

a. hepatotoxic b. neurotoxic c. cardio toxic d. nephrotoxic

112- A 12 year-girl with a 2 year history of epilepsy was admitted to the Emergency department after she swallowed a large amount of her medical tablets, she had lapsed into coma after complaining for 2 hours of dizziness, slurred speech, and unsteadiness of gait. Her temperature was 36°c, blood pressure was 85/50. What is the most probable drug of the following?

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a) Phenobarbital b) Acetaminophen c) Aspirin d) Atropine

113-The primary mechanism of action of phenytoin is:

A) Opening potassium channels

B) Blocking voltage-gated sodium channels in the inactive state

C) Direct stimulation of GABA receptors

D) Inhibition of calcium channels only

114-Which of the following is a feature of oral phenytoin toxicity?

A) Miosis B) Nystagmus and ataxia C) Severe hypertension D) Diarrhea

115-Hyperglycemia in phenytoin toxicity is due to:

A) Increased insulin secretion B) Increased glucose excretion

C) Inhibition of insulin release D) Decreased glucagon

116-Rapid IV administration of phenytoin may lead to:

A) Renal failure B) Seizures C) Liver failure D) Cardiac arrest

117-The maximum recommended rate for IV phenytoin infusion is:

A) 10 mg/min B) 25 mg/min C) 50 mg/min D) 100 mg/min

118-Seizures in phenytoin toxicity are treated with:

A) Phenytoin B) Benzodiazepines C) Atropine D) Morphine

119-Carbamazepine is structurally related to:

A) Tricyclic antidepressants B) Barbiturates C) Benzodiazepines D) Opioid

120-Which of the following is a use of carbamazepine?

A) Parkinson's disease B) Trigeminal neuralgia C) Asthma D) Hypertension

121-A characteristic feature of carbamazepine toxicity is:

A) Persistent coma without fluctuation B) Bradycardia only

C) Severe hypoglycemia D) Fluctuating level of consciousness)

122-Phenytoin decreases neuronal excitability mainly by:

A) Increasing potassium efflux B) Enhancing dopamine release

C) Reducing sodium influx into neurons D) Blocking chloride channels

123-Which of the following is an indication for oral phenytoin?

A) Status epileptics B) Grand mal epilepsy C) Acute anxiety D) Migraine attack

124-Which of the following is a gastrointestinal effect of phenytoin toxicity?

A) GIT irritation B) Constipation C) Peptic ulcer D) Severe diarrhea

125-The CNS effect of severe phenytoin toxicity includes:

A) Euphoria B) Hallucinations only C) Stupor and coma D) Hyperactivity

126-During IV phenytoin therapy, monitoring should include:

A) Liver enzymes only B) Blood glucose only

C) Renal function only D) ECG and blood pressure

127-Carbamazepine acts mainly on:

A) Calcium channels B) Sodium channels C) Potassium channels D) GABA receptors

128-Which of the following is NOT a typical use of carbamazepine?

A) Epilepsy B) Trigeminal neuralgia

C) Manic depressive illness D) Acute bacterial infection

129-A patient with carbamazepine toxicity may show:

A) Severe hypertension B) Tachycardia and hypotension

C) Hyperglycemia D) Miosis

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130-Anticholinergic manifestations in carbamazepine toxicity include:

A) Salivation and sweating B) Bradycardia

C) Dry mouth and tachycardia D) Diarrhea

131-In children, carbamazepine toxicity is more likely to cause:

A) Dystonic reactions and seizures B) Renal failure

C) Severe hypoglycemia D) Liver cirrhosis

132-The most common electrolyte imbalance in acute digitalis toxicity is:

a. Hypocalcemia. b. Hypokalemia. c. Hypercalcemia. d. Hyperkalemia

133-The initial acid base-derangment in mild Aspirin toxicity is usually:

a. Respiratory acidosis. b. Metabolic acidosis

c. Respiratory alkalosis. d. Metabolic alkalosis.

134-Toxic causes of hypokalemia include:

a. Datura Poisoning. b. Acute digitalis toxicity.

c. Theophylline poisoning d. Iron overdose.

135-The mechanism of action of barbiturate is:

a. Stimulation of Acetylcholine receptors. b. Stimulation of Glutamate receptors.

c. Stimulation of GABA receptors. d. Stimulation of GR receptors.

136-Benzodiazepines act on CNS through:

a) Increasing catecholamine. b) Increasing the activity of GABA.

c) Decreasing catecholamine. d) Decreasing the activity of GABA.

137-Retinal bleeding, tinnitus, gastric ulcer and skin eruption are chronic toxic manifestations of:

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a. Carbamazepine. b. Salicylate. c. Mercury. d. Methanol.

138-A patient presented with tinnitus, weight loss & haematemesis may be a case of chronic poisoning with:

a. Morphine. b. Aspirin. c. Arsenic. d. Phenobarbitone. e. Lead.

139-Metabolic acidosis is the most important feature of poisoning by:

a. Oxalic acid. b. Carbolic acid. c. Acetyl salicylic acid. d. Hydrocyanic acid.

140-Gastric ulceration & bloody vomitus occur in cases of poisoning by:

a. Amphetamine. b. Acetyl salicylic acid. c. Benzodiazepines. d. a & b only.

141-Drug automatism leading to repeated intake & acute toxicity of the following drug:

a. Aspirin. b. Amphetamine. c. Barbiturates. d. Acetaminophen.

142-Flumazenil is used as an antidote in case of poisoning with:

a. Cocaine b. Diazepam c. Phenobarbitone.

d. Methanol e. Acetaminophen.

143-Severe depression of vasomotor center occurs in case of poisoning with:

a. Atropine. b. Aspirin. c. Kerosene.

d. Paracetamol. e. Barbiturates.

144-Both hepatic & renal damages occur in poisoning with:

a. Paracetamol b. Zink phosphide. c. Ethanol.

d. Atropine. e. Morphine

145-Paracetamol is metabolized into a toxic metabolite by:

a. Conjugation with sulphates.

b. The action of cytochrome P450 oxidase enzyme system.

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c. Conjugation with glucouronic acid. d. Conjugation with glutathione.

e. None of the above.

146-Which of the following is correct about barbiturates?

a. They are synthetic drugs. b. They are sedative hypnotics.

c. They are barbituric acid derivatives.

d. They are one of the commonest causes of poisoning. e. All of the above

147-Barbiturate coma is characterized by:

a. Intense cyanosis and Slow shallow stertorous breathing.

b. Slightly dilated but reactive pupils.

c. Skeletal muscle relaxation with decrease of reflexes.

d. Manifestations of shock due to medullary depression. e. All of the above.

148-Which of the following methods of enhancement of elimination is effective in barbiturate poisoning?

a. MDAC. b. Hemodialysis. c. Hemoperfusion.

d. Forced alkaline diuresis e. All of the above.

149-The most significant finding in arterial blood gases analysis in case of acute salicylate poisoning is:

a. Metabolic acidosis b. Metabolic alkalosis. c. Respiratory acidosis.

d. None of the above. e. All of the above.

150-Gastric lavage in case of acute salicylate poisoning is permitted up to:

a. One hour. b. Two hours. c. Three hours

d. Four hours e. Twelve hours

151-Which of the following organs may be damaged in acute acetaminophen toxicity?

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a. Liver. b. Brain. c. Eye. d. Lung e. Ear.

152-Classical acute acetaminophen toxicity is characterized by:

a- Four stages. b- Three stages c-Two stages.

d-One-stages e-Five stages.

153- The second stage in acute acetaminophen toxicity is characterized by:

a- Abnormalities of the liver functions. b-Right upper quadrant pain and tenderness.

c- Bleeding tendencies due to coagulation defects. d- Jaundice. e- Hematuria.

154-The third stage in acute acetaminophen toxicity is characterized by:

a. Peak abnormalities of the liver functions. b. Right upper quadrant pain and tenderness.

c. Bleeding tendencies due to coagulation defects. d. All of the above

e. None of the above.

155-The toxicity of acetaminophen is mainly due to:-

a. Sulfation b. Glutathione excess. c. Glucuronide conjugation

d. Toxic metabolite which is produced by cytochrome P450 oxidase enzyme system.

e. Excretion in urine.

156-Benzodiazepines act on the CNS through the following mechanism:

a. Stimulating the inhibitory neurotransmitter (GABA) "Gamma amino butyric acid"

b. Inhibiting the inhibitory neurotransmitter (GABA) "Gamma amino butyric acid

c. Increasing catecholamines. d. Increasing serotonin. e. None of the above

157-Which of the following is the specific antidote in case of acute benzodiazepine toxicity?

a. Flumazenil. b. FAB. c. BAL D. EDTA. e. DMSA

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158-N-acetylcystein 20% is the specific antidote of

a. Acetaminophen. b. Aspirin. c. Barbiturates.

d. Benzodiazepines. e. None of the above.

159-All the followings are true for barbiturate coma Except:

a. Deep coma. b. Low blood pressure. c. Constricted pupils

d. Slow shallow respiration.

160-The metabolic acidosis occurring with acute salicylate toxicity is due to:

a. Increase lactic acid. b. Increase ketone bodies. c. Increase organic acids.

d. Increase amino acids. e. All of the above

161-in mild salicylate toxicity, the blood salicylate level ranges from:

a. 100 - 120mg / d L b 10 - 20mg / d L c. 45 - 65mg / dL

d. 65-90 mg/dL e. < 20mg / dL

162-In moderate salicylate toxicity, the blood salicylate level ranges from:

a. 100 - 120mg / dL b. 10 - 20mg / d L c. 4S - 65mg / dL

d. 65 - 90mg / dL c. < 20mg / dL

163-In sever toxicity, the blood salicylate level ranges from:

a. 100 - 120mg / dL b - 20mg / dL c. 45 - 65mg / dL

d. 65 - 90mg / dL e. 90 -120 mg /dL

164-Annexate is the specific antidote for:

a. Paracetamol b. Salicylate. c. Amphetamine

d. Benzodiazepines. e. Barbiturate.

165-The following poison produces jaundice:

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a. Lead b. Cocaine. c. HCN. d. Paracetamol e. Opium.

166-The following poison produces bullae:

a. Lead b. Cocaine. c. HCN. d. barbiturates e. Opium

1. B	2. A	3. B	4. A	5. A
6. B	7. C	8. C	9. D	10.A
11.C	12.D	13.D	14.A	15.C
16.C	17.A	18.B	19.B	20.B
21.D	22.A	23.B	24.B	25.B,D
26.C	27.B	28.C	29.B	30.D
31.A	32.C	33.B	34.B	35.C
36.B	37.A	38.B	39.C	40.C
41.D	42.C	43.B	44.D	45.C
46.B	47.D	48.B	49.B	50.B
51.B	52.D	53.A	54.C	55.D
56.A	57.B	58.D	59.D	60.C
61.A	62.A	63.B	64.A	65.B
66.A	67.B	68.B	69.A	70.B
71.A	72.B	73.A	74.B	75.B
76.A	77.B	78.A	79.B	80.A
81.A	82.A	83.B	84.A	85.A
86.B	87.A	88.B	89.B	90.B
91.A	92.A	93.B	94.B	95.A
96.A	97.A	98.B	99.A	100. A
101. B	102. A	103. A	104. B	105. A
106. C	107. C	108. C	109. D	110. B
111. A	112. A	113. B	114. B	115. C
116. D	117. C	118. B	119. A	120. B
121. D	122. B	123. C	124. A	125. C
126. D	127. B	128. D	129. B	130. C
131. A	132. D	133. C	134. C	135. C
136. B	137. B	138. B	139. C	140. B
141. C	142. B	143. E	144. A	145. B
146. E	147. E	148. E	149. A	150. E
151. A	152. A	153. A	154. D	155. D
156. A	157. A	158. A	159. C	160. E

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161.	C	162.	D	163.	E	164.	D	165.	D
166.	D	167.		168.		169.		170.	

Gaseous poisons

1-Carbon monoxide mechanism of action? {Final2025}

A-Has a higher affinity to HB than oxygen

B-Shift oxyhemoglobin dissociation curve to the right

2-Which of the following statements is true about carbon monoxide: {Qena}

a) Its long term sequelae is due to damage of cerebral cortex

b) It is heavier than air

c) It has an affinity to hemoglobin 20 times greater than 02

d) It shifts the oxyhemoglobin dissociation curve to the left

3-Carbon monoxide is a natural byproduct of the breakdown of protoporphyrin to: {Formative2024}

a) Glucose b) Bilirubin c) Oxygen and water d) Glutathione

4-SLUD toxidrome include the following except: {Formative2024}

a) Sweating b) Urination c) Lacrimation d) Mydriasis

5- CO has affinity to hemoglobin times more than O2 {Final2023}

A- 200-250 time B- More than 300 C- 50-100 time D- 15

6- CO binds to the following compounds EXCEPT: {Final2021}{kasr}

a. Hemoglobin b. Myoglobin c. Alcohol dehydrogenase d. Cytochrome oxidase

7- 12 years old male child has been found in the bathroom with conscious level, his carboxyhem level is 50%, the most important for treatment {Mid2022}

A- Hyperbaric oxygen B- Nitrites C- Mechanical ventilation D- 100% oxygen

8-Hyperbaric oxygen is indicated in {Formative2022}

a) Pregnancy with CoHb=15% or more b) CoHb level is 40% or more

c) A&B d) Metabolic alkalosis

9-Toxin that has rotten egg odor {Formative2022}

a) Co b) Hydrogen sulphide c) Aluminum phosphide d) None of the above

10- a woman with a history of running of car engine in garage and pulse was 110 beats/min, blood pressure 90/60 mmHg with metabolic acidosis? {Mid2024}

A. CO poisoning B. Cyanide poisoning

11-All the following are muscarenic manifestations of organophosphate poisoning except {Qena}

a) Lacrimation. b) Muscle fasciculation. c) Salivation. d) Diaphorisis

12-Sewage worker presented to the hospital with disturbed conscious level. Examination revealed hypotension 90/60 mmHg and pulmonary edema. He is mostly intoxicated with: {Qena}

a-Co b-Cyanide c-hydrogen sulphide d-organophosphorus

13- Tile first aid management in acute CO toxicity is: {Qena}

a.O2 100% b. Hyperbaric oxygen. c. Fresh air. d Endotracheal tube

14-In carbon monoxide poisoning, hyperbaric oxygen causes the following {Qena}

a-Shifting of oxygen dissociation curve to right.

b- Shifting of oxygen dissociation curve to left.

c- Increase of oxygen concentration in plasma.

d- A&C

15- Co poisoning causes red asphyxia {قديم}

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A- True B- False

16- Co gas is considered as a silent killer {قديم}

A- True B- False

17-the following has characterized smell except: {Qena}

a-phenol. b-alcohol. c-Carbon monoxide. d-phosphorus.

18-In carbamate toxicity, the associated complication is: {Qena}

a. Intermediate syndrome b. Delayed neuropathy

c. Cardiotoxic effect d. None of the above

19-Carbon monoxide is characterized by all the following, EXCEPT; {Qena}

A)Colorless B) Irritating C) Odorless D) Hypoxic gas

20-The endogenous source CO gas is: {kasr}

a. Protein catabolism b. Hemoglobin catabolism

c. Lipid catabolism d. Carbohydrates-catabolism

21-All the following poisons have characteristic smell, except: {kasr}

a. Organophosphorus. b. Carbon monoxide. c. Cyanide d. Opium.

22-CO poisoning causes: {kasr}

a. Shift of 02 dissociation curve to the right.

b. Shift of O2 dissociation curve to the left.

c. All of the above d. None of the above.

23-The red color in CO poisoning is due to {kasr}

a. Oxy hemoglobin b. Carboxy hemoglobin.

c. Methemoglobin. d. Hemochromagen.

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24-Post mortem cherry red discoloration is due to {kasr}

a. Asphyxia. b. Carbon monoxide poisoning

c. Drowning d. Phosphorus poisoning

25-Carbon monoxide is rapidly fatal when its percentage in the air is: {kasr}

a. 0.001%. b. 0.01% c. 0.1%. d. 1.00%

26-The affinity of CO for Hb is greater than that for 02 by a factor of: {kasr}

a. 100. b. 150. c. 210. d. 2100.

27-All the following have characteristic smell, except: {kasr}

a. Phenol. b. Yellow Phosphorus. c. Ethanol. d. Carbon monoxide.

28-Carbon monoxide poisoning produces the following type of anoxia: {kasr}

a. Anemic anoxia b. Anoxic anoxia. c. Stagnant anoxia. d. Histotoxic anoxia.

29-In CO poisoning, hyperbaric 02 causes the following except: {kasr}

a. Acceleration of association of 02 to Hb. b. Shifting of 02 dissociation curve to right.

c. Shifting of 02 dissociation curve to left. d. Increase of 02 concentration in plasma.

30-The following are complications in CO poisoning, except: {kasr}

a. Amnesia. b. Renal failure. c. Pulmonary edema. d. Peptic ulcer.

31-Toxic coma secondary to hypoxia occurs in: {kasr}

a. CO poisoning b. Methanol toxicity. c. Ethanol toxicity d. Salicylate toxicity

32-Co gas has a characteristic smell. {kasr}

a. true b. false

33-The affinity of CO for Hb is greater than that for O2 by about 210 times. {kasr}

a. true b. false

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34-Hyperbaric oxygen is more highly beneficial than 100% oxygen in animal models of cyanide poisoning. {kasr}

a. true b. false

35-Carbon monoxide most commonly generated due to one of the following:

a. Methylene chloride converted inside the body

b. breakdown of hemoglobin and myoglobin

c. Incomplete combustion of carbon containing materials

d. During firefighting

36-Mitochondrial impairment in carbon monoxide poisoning is mainly due to affection of

A. Xanthine dehydrogenase. B. Xanthine oxidase.

C. Cytochrome oxidase. D. Superoxide dismutase

37-The most common complication of the Hyperbaric oxygen is

a. Barotrauma of the tympanic membrane b. Rupture of the cornea

c. Barotrauma of the stomach d. Rupture of pulmonary artery

38-Most common mode of poisoning of organophosphorus is:

a. Homicidal b. Suicidal c.Accidental d. Criminal

39-One of the mechanisms of CO toxicity is:

a. Shifting of oxygen dissociation curve to right. b. Binding to hemoglobin in blood reservoirs.

c. Impairment of cytochrome oxidase. d. Increasing oxygen carrying capacity of hemoglobin.

40-Which statement is true as regards hyperbaric oxygen treatment in CO poisoning?

a. It is used in mild CO poisoning cases.

b. It decreases COHb half-life to 30 minutes.

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c. It increases the amount of dissolved oxygen in plasma.

d. It decreases COHb half-life to 40 minutes.

41-Hyperbaric O, 100% inhalation is essential in treatment of acute poisoning with:

a. Ethanol b. Methanol. c. Kerosene.

d. Carbon monoxide. e. Cyanide.

42-Mechanism of toxicity in CO poisoning includes:

a. Formation of carboxyhemoglobin. b. Formation of oxyhemoglobin.

c. Shift of O2 dissociation curve to the right. d. a & c. e. None of the above.

43-Characteristic smell can be found in all Except:

Opium poisoning. b. Phenol poisoning. c. CO poisoning.

d. Kerosene poisoning. e. Cyanide poisoning.

1. A	2. D	3. B	4. D	5. A
6. C	7. A	8. C	9. B	10.A
11.B	12.C	13.C	14.D	15.A
16.A	17.C	18.D	19.B	20.B
21.B	22.B	23.B	24.B	25.D
26.C	27.D	28.A	29.C	30.D
31.A	32.B	33.A	34.B	35.C
36.C	37.A	38.C	39.C	40.C
41.D	42.A	43.C	44.	45.

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Volatile poisons

1-Methanol charactersied by {Final2025}

A-Metabolic acidosis with wide anion gap

2-Hydro cyanide acts on {Final2025}

A-Cytochrome oxidase enzyme

3-The antidote of methanol is ethanol:- {Formative2022}

a) True b) False

4-Corneal glistening with dilated fixed pupil is one symptom of toxicity by:- {Formative2022}

a) Carbone monoxide b) Cyanide c) Phenol d) Methanol

5- Ethyl alcohol toxicity manifested with all of the following EXCEPT {Final2022}{Final2023}

A- Flushed face B- Vomiting C- Elevated temperature D- Blurring of vision

6- Cause of death in kerosene toxicity {Mid2024}

A. Early due to CNS depression, late due to pneumonia

7- a 5-year-old child came to hospital with crepitations over chest claimed to ingest clear fluid with characteristic odor with vomiting, normal pupils, shallow respiration, B P 110/70, pulse 70 b/m. What's your diagnosis? {Final2023}

A- kerosene b) organophosphorus c) ethanol d) sulphuric acid

8-Ethyl alcohol is: {Mid2021}

a. Stimulant b. Depressant c. Hallucinogen d. Sedative

9-Hemodialysis is used for treatment of ethyl alcohol if its level exceeds: {Final2021}

a. 0.3% b. 0.4% c. 0.5% d. 0.6%

10-Treatment modalities of kerosene poisoning are: {Final2021}{Formative2024}

a. Emesis and steroids b. Gastric lavage and steroids

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c. Emesis and oxygen d. Gastric lavage and atropine

11-The following drugs cause hyperthermia except: {Formative2022}

a) cocaine b) Anticholinergic c) salicylate d) ethanol

12- Methyl alcohol toxicity causes all of the followings except {Mid2024}

A. Optic damage B. Metabolic alkalosis C. High anion gap D. CNS depression

13- Which of the following vitamin can be used as antidote in cyanide poisoning: {قديم}{Qena}

A- Vitamin B6 B- Vitamin B12 C- Vitamin A D- Vitamin C

14- Cyanide poisoning causes brown asphyxia {قديم}

A- True B- False

15- Methanol poisoning is more dangerous because of the following except {قديم}

A- Blindness B- small fatal dose

C- rapid accumulation of acetaldehyde D- Sever metabolic acidosis

16- Methanol toxicity causes severe metabolic acidosis {قديم}

A- True B- False

17-The principle of management of methanol poisoning is: {Qena}

a. Elimination of methanol substance. b. Induction of methanol metabolism.

c. Prevention of Methanol metabolism. d. Enhancing excretion of methanol metabolites.

18-Induction of methemoglobinemia is needed in treatment {Qena}

a) Ethanol b) Methanol c) Cyanide d) Kerosene

19-The following is effective in management of acute alcohol intoxication: {Qena}

a. Enzyme inducer. b. Co enzymes. c. Trace elements. d. Enzyme inhibitor.

20-Euphoria in ethyl alcohol poisoning is due to: {Qena}

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a. cortical stimulation b. cortical inhibition c, analgesia d. all of the above

21-Management of Kerosene poisoning includes all the following except: {Qena}

a- Oxygen. b- Activated charcoal. C- Antibiotics. d- Corticosteroids.

22- In kerosene poisoning, stomach is best evacuated by {قديم}

A- Syrup of ipecac B- Stomach tube only C- Apomorphine D- None of the above

23-Gastric lavage can be done up to 12 hours post-ingestion in the following poisons, except: {kasr}

A. Barbiturates. B. Salicylates. C. Ethanol D. Morphine.

24-Ethanol is absorbed from all the following parts, except: {kasr}

a. Skin. b. Stomach. c. Small intestine. d. Large intestine.

25-One of the following statements is incorrect regarding ethanol: {kasr}

a. It is distributed to all tissues according to the water content.

b. 80% are absorbed from stomach and large intestine.

c. It is metabolized mainly by oxidation.

d. It can be detected in tears, bile and gastric juice.

26-Ethyl alcohol intoxication leads to all the following, except: {kasr}

a. Hypoglycemia. b. Hyperthermia

c. Accumulation of fat in liver. d. Accumulation of lactic acid.

27-Blood alcohol concentration in an individual with hiccough and slurred speech is: {kasr}

a. 10-50 mg% b. 50 - 150mg % c. 150 - 300mg % d. > 300mg %

28-Car accidents and social crimes are common when blood alcohol level ranges between: {kasr}

a. 10 - 50mg % b. 50-150 mg% c. 150-300 mg% d. >300 mg%

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29-A dead person due to alcohol intoxication will have all the following, except: {kasr}

a. External and internal signs of asphyxia. b. Blood alcohol concentration > 500 mg%

c. Characteristic smell. d. Pale blue hypostasis

30-Alcohol toxicity is characterized by: {kasr}

a. Constricted fixed pupils. b. Increased acuity of special senses.

c. Inhibition of mental brakes. d. False cold sensation.

31-All of the followings are used to treat a patient with alcohol toxicity, except: {kasr}

a. Gastric lavage with NaHCO3 b. Hemodialysis.

c. Activated charcoal. d. Vitamin B

32-Aldehyde dehydrogenase acts on {kasr}

a. Formic acid. b. Formaldehyde. c. Acetaldehyde d. Both B & C.

33-Methanol is more toxic than ethanol due to: {kasr}

a. Formation of formic acid. b. Slower rate of excretion.

c. More profound metabolic acidosis. d. All of the above.

34-Leucovorin treats methanol toxicity by: {kasr}

a. Inhibition of alcohol dehydrogenase

b. Conversion of formic acid to Co2, & H2o

c. Competition for alcohol dehydrogenase

d. Inhibition of metabolism of methanol to toxic

35-Blocking of methanol metabolism to its toxic metabolites is a using: {kasr}

a. Ethanol. b. Folic acid. c. Methyl pyrazole. d. Both A & C

36-The target organ(s) in kerosene toxicity is (are): {kasr}

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a. GIT b. CNS c. Lungs d. All of the above.

37-All of the following manifestations may occur with hydrocarbon aspiration except: {kasr}

a. Dyspnea b. Cyanosis. c. Hypothermia. d. Pulmonary edema.

38-The most effective line of treatment in cyanide toxicity is: {kasr}

a. Fresh blood transfusion b. Hyperbaric 02

c. Artificial respiration d. Nitrite thiosulfate therapy

39-Cyanide poisoning produces the following type of anoxia: {kasr}

a. Anemic. b. Anoxic c. Histotoxic d. Stagnant

40-Coma due to cyanide poisoning is characterized by the following, except: {kasr}

a. Cyanide cry b. Constricted pupil c. Bitter almond smell. d. Clench jaw.

41- One of the following vitamins may be used as an antidote to cyanide poisoning: {kasr}

a. Vitamin B6. b. Vitamin B12a c. Vitamin B12. d. Vitamin B1.

42-The glycoside amygdalin is a source of poisoning by: {kasr}

a. Hyoscine. b. Atropine. c. Strychnine. d. Cyanide

43-Inactivation of cytochrome oxidase enzyme occurs in case of: {kasr}

a. Atropine b. Ethanol. c. Cyanide. d. Phenol.

44-In case of ingestion of cyanide, the first treatment is: {kasr}

a. Artificial respiration and O_2 inhalation. b. Stomach wash.

c. Administration of nitrites. d. Injection of methylene blue.

45-In cyanide poisoning, nitrites are given in order to: {kasr}

a. Reduce the cyanide. b. Oxidize the cyanide.

c. Reduce oxyhemoglobin d. Produce methemoglobinemia

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46-All the following are characters of cyanide, except: {kasr}

a. A colorless liquid b. A volatile liquid

c. A white liquid. d. It has a bitter almond oils

47-Fatal period of cyanide poisoning is: {kasr}

a. I hour. b. 3 hours. c. 10 minutes. d. 45 minutes

48-All the following are local antidotes in cyanide poisoning, excepts: {kasr}

a.H2O2 b. Na thiosulfate. c. Charcoal d. All of the above

49-Dilated fixed glistening pupils occur in case of poisoning with: {kasr}

a. Opium. b. Ethanol. c. Cyanide. d. Phenol

50-Successful treatment of cyanide poisoning is indicated by the presence of: {kasr}

a. Cyanomethemoglobin. b. COHb c. Reduced Hb. d. Oxy Hb.

51-Acidosis is a common finding in all of the following poisoning except: {kasr}

a. Methanol poisoning b. Salicylate poisoning

c. Iron poisoning d. Oxalic acid poisoning

52-Cyanide is more tightly bound to iron in {kasr}

a. ferric state b. ferrous state c. ferrous carbonate state d. ferrous acetate

53-Ethanol is more toxic than methanol. {kasr}

a. true b. false

54-Both ethanol and 4-methyl pyrazole (4-MP) do the same action, but (4-MP) is preferred. {kasr}

a. true b. false

55-Vitamin B6 is used in treatment of methanol toxicity. {kasr}

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a. true b. false

56-Hemodialysis is ineffective in treatment of ethanol intoxication. {kasr}

a. true b. false

57-Breath analyzer is used by the mobile police. {kasr}

a. true b. false

58-Metabolic acidosis is caused by methanol but not by ethanol toxicity. {kasr}

a. true b. false

59-Death may occur when ethanol concentration is > 500 mg%. {kasr}

a. true b. false

60-Methanol causes profound metabolic acidosis. {kasr}

a. true b. false

61-Methanol overdose causes ocular toxicity. {kasr}

a. true b. false

62-The target organs of hydrocarbons are primarily the lungs and CNS. {kasr}

a. true b. false

63-Absorbed cyanide is best chelated by Calcium disodium EDTA. {kasr}

a. true b. false

64-Patient dies in few minutes from anemic anoxia in HCN poisoning. {kasr}

a. true b. false

65-The glycoside amygdalin is a source of poisoning by Hyoscine. {kasr}

a. true b. false

66-Inactivation of cytochrome oxidase enzyme occurs in case of Cyanide. {kasr}

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a. true b. false

67-Dilated fixed glistening pupils occur in case of poisoning with opium. {kasr}

a. true b. false

68-The gas used in gas chambers for execution of murders is carbon monoxide. {kasr}

a. true b. false

69-Infusions of large volumes of hydroxycobalamin are required so that it can be used as an antidote for cyanide poisoning. {kasr}

a. true b. false

70-Ethyl alcohol is used as an antidote of

a. Kerosene b. Methanol c. Barbiturate d. organophosphorous

71-A 25-year-old female was brought by her family to the hospital with a history of drinking kerosene in attempt to suicide. She was comatose and once arrived she died. The immediate cause of death in this case is

A. Pneumonia. B. Renal failure. C. Metabolic Acidosis. D. Respiratory failure

72-The main target organ in kerosene toxicity is

A. Lungs. B. Heart. C. Kidney. D. Spleen.

73-The following poison produces red coloured skin:

a. Lead. b. Cocaine. c. HCN. d. Paracetamol.

74-Regarding to kerosene poisoning, delayed cause of death is mainly due to:

a) Respiratory failure b) Heart failure c) Pneumonia d) Chemical pneumonitis

75-A 4 years child ingested a fluid, vomited twice & complaining of cough & tachypnea. After 24 hours his temp becomes 39 °C due to bronchopneumonia. The possible diagnosis is

a. phenol b. kerosene c. methanol d. caustic potash

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76-The aim of treatment of methanol poisoning is to:

a. Oxidize it to formaldehyde. b. Oxidize it to acetaldehyde.

c. Prevent its oxidation. d. Form acetic acid.

77-In hydrocarbon ingestion, one of the following suggests aspiration:

a. Dyspnea b. Persistent cough c. Hypoxia. d. Cyanosis.

78-In methyl alcohol poisoning, there is central nervous system depression, cardiac depression and optic nerve atrophy. These effects are produced due to:

a. Formaldehyde and formic acid b. Acetaldehyde c. Pyridine d. Acetic acid

79-A Person with blood ethanol above 0.5% is most likely to be:

a. Dead. b. Euphoric. c. Comatose

d. Slightly drunk. e Heavily drunk.

80-Ethanol is mainly metabolized by:

a. The liver b. The kidneys c. The lungs

d. All of the above. e. None of the above.

81-Gastric lavage in acute ethanol poisoning is done by

a. Sodium bicarbonate b. Methanol. c. Tannic acid

d. Sodium thiosulphate. e. Calcium hydroxide.

82-Vitamin B6 helps to accelerate the metabolism of:

a. Methanol b. Ethanol. c. Kerosene.

d. a & b e. None of the above.

83-Antidotes used in treatment of acute methanol poisoning include:

a. Ethanol. b. Thiamine. c. Folinic acid

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d. All of the above. e. a & c only.

84-Toxic effects of methanol include:

a. Respiratory center depression. b. Optic nerve atrophy. c. Metabolic acidosis.

d. All of the above. e. b&c only

85-Chemical pneumonia most common after poisoning by:

a. Ethanol b. Kerosene. c. Methanol

d. Phenol. e. Sulphuric acid.

86-In cases of big amount of potassium cyanide ingestion, the first line of treatment is:

a- Oxygen inhalation. b- Gastric lavage. c- Administration of nitrites.

d-Blood transfusion. e- Injection of methylene blue.

87-The presence of cyanomethaemoglobin in a patient after cyanide poisoning indicates:

a- Severe toxicity. b- Dangerous anoxia. c- Starting haemolysis.

d- Successful treatment. e- Need for blood transfusion.

88-The toxicity of amygdaline is due to:

a- Central anoxia. b- Tissue anoxia. c- Anaemic anoxia.

d- Stagnant anoxia. e. Hypoxic anoxia.

89-The absorbed cyanide can be treated by:

a- Calcium gluconate. b- Ferric chlorid c- Ferrous sulphate

d- Cobalt editate. e- Magnesium sulphate.

90-Vitamin B12a 1000 µg I.V. is used in treatment of:

a. Ammonium hydroxide poisoning. b. Carbolic acid poisoning.

c. Caustic potash poisoning. d. Cyanide poisoning.

e. Oxalic acid poisoning.

91-The toxicity of hydrocyanic acid is due to:

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a. Hypoxic hypoxia. b. Tissue (histotoxic) hypoxia.

c. Anemic hypoxia. d. Stagnant hypoxia.

92-Hemodialysis is indicated when ethanol concentration is > 120 mg%. {kasr}

a. true b. false

1. A	2. A	3. A	4. B	5. C
6. A	7. A	8. B	9. B	10.B
11.D	12.B	13.B	14.B	15.C
16.A	17.C	18.C	19.D	20.B
21.B	22.D	23.C	24.A	25.B
26.B	27.C	28.B	29.D	30.C
31.C	32.D	33.D	34.B	35.D
36.D	37.C	38.D	39.C	40.B
41.B	42.D	43.C	44.C	45.D
46.C	47.C	48.C	49.C	50.A
51.D	52.A	53.B	54.A	55.B
56.B	57.A	58.B	59.A	60.A
61.A	62.A	63.B	64.B	65.B
66.A	67.B	68.B	69.A	70.B
71.D	72.A	73.C	74.C	75.B
76.C	77.B	78.A	79.A	80.A
81.A	82.B	83.E	84.D	85.B
86.C	87.D	88.B	89.D	90.D
91.B	92.B	93.	94.	95.

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Drug dependence

1-contraindication in cocain and amphetamine toxicity? {Final2025}

A-beta blocker B-nitroglycerin C-aspirine D-benzodiazepines

2-Tolerance is responsible for {Final2025}

A-Physical deterioration B-Cravings

C-Withdrawal symptoms D-Physical and Mental deterioration

3-Morphine inhibit the following centers except: {Formative2024}

a) Vasomotor center b) Heat regulating center

c) Respiratory center d) Vomiting center

4- Non medical use of drugs that may not necessarily involve dependence but cause harm to the user? {Mid2024}

A. Drug abuse B. Misuse C. Drug dependence D. Habituation

5- Patient came with cocaine pug, Irritability, increase aggressiveness the best treatment is {Mid2024}

A. Abrupt withdrawal B. Gradual withdrawal C. Substitution therapy

6- Psychological dependence cause {Final2023}

A- habituation B- Withdrawal manifestation C- Craving and relapse d) Tolerance

7- Tramadol is synthetic analogue of {Final2023}

A- codeine b) Morphine c) Atropine

8-Feeling as if insects are creeping under the skin occur in : {Formative2022}

a) Cocaine b) Cannabis c) Barbiturate d) All of the above

9-Sudden withdrawal is preferred in the treatment of chronic toxicity except in: {Formative2022}

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a) Amphetamine b) Cannabis c) Morphine d) Non of the above

10-Tolerance responsible for {Mid2021}

a. Physical and moral deterioration b. Craving

c. Withdrawal manifestations d. Relapse

11-Tolerance is: {Final2021}

a. Abnormal drug response b. Exaggerated drug response

c. Diminished drug response on repeated usage d. None of the above.

12-A 26-year-old male who has developed a long-standing addiction to heroin which began 4 years earlier while travelling. He visited you for advice. He has tried going cold turkey on number of occasions but developed unacceptable restlessness, anxiety, vomiting, diarrhea. He now has a child and decided to stop. Which one of the following is the most appropriate prescription with respect to medically managing withdrawal? {Final2021}

a. Buprenorphine b. Methadone. c. Diazepam d. Dihydrocodeine

13- A 20-year-old male addict on Amphetamines hyperexcitability, tachycardia. And tremors. The doctor advised him to use {Mid2022}{Final2022}

A- beta blocker to decrease tachycardia.

B- Methadone to decrease withdrawal signs.

C- Benzodiazepines

D- Gradual withdrawal by decrease the dose of amphetamine gradually.

14-Abrupt withdrawal is used in treatment of dependence of the following except: {Qena}

a. barbiturate. b. alcohol. c. Cannabis. d. amphetamine

15-The moral deterioration in an addict is due to: {Qena}

a. Psychological dependence. b. Physical dependence. c. Tolerance. d. Overdosage.

16-Gradual withdrawal is used in treatment of dependence of the following except {Qena}

a. barbiturate. b. morphine. c. Cannabis.

17-The withdrawal manifestations in an addict are due to: {قديم}{Qena}

a. Psychological dependence b. Physical dependence c. Tolerance d. Overdosage

18-Abrupt withdrawal is used in treatment of dependence of the following: {Qena}

a. barbiturate. b. morphine. c. Cannabis. d.heroin

19-The following drugs are CNS stimulants, except: {kasr}

a. Amphetamine b. Benzodiazepines. c. Cocaine. d. Theophylline.

20-All the following are characters of drug dependence, except: {kasr}

a. Physical& psychological dependence. b. Repeated use of a drug for non-medical reason.

c. It is taken by snuffing& injection only. d. Mood changes& a state of well being

21-Tolerance in drug dependence is due to all the following mechanisms except: {kasr}

a. Decreased absorption. b. Decreased excretion.

c. Increased metabolism. d. Formation of antibodies against the drug

22-Addicts need to increase the dose of abused drug continuously due to: {kasr}

a. Increased metabolism & decreased excretion.

b. Decreased absorption& decreased excretion.

c. Formation of an alternative metabolic pathway & sensitization of receptors.

d. Increased excretion& desensitization of receptors.

23-Dependence is characterized by all the following, except: {kasr}

a. Detrimental effect on the individual& society.

b. Intolerance.

c. The cells become dependent on the drug

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d. Sudden withdrawal may be life threatening.

24-Psychic dependence: {kasr}

a. Means inability of body cells to function normally without the drug.

b. Is responsible for occurrence of withdrawal symptoms.

c. Is satisfaction on administration of a certain agent.

d. All of the above.

25-Medical hazards of addiction include all the following, except: {kasr}

a. Sexually transmitted diseases. b. Drug overdose c. AIDS. d. DIC

26-Regarding the withdrawal manifestations of opiates, all the following statements are true, except: {kasr}

a. They start immediately after naloxone injection.

b. They remain severe for 10 days, and then subside gradually.

c. They are usually the opposite of the drug effects.

d. They are stopped by taking opiates.

27-Withdrawal symptoms of opiates, do not include: {kasr}

a. Yawning. b. Increased secretions.

c. Hot sensation. d. Cardiovascular collapse.

28-The following drug is used in treatment of opiate addiction, to stimulate the release of endogenous opioids: {kasr}

a. Clonidine. b. Naltrexone. c. Methadone. d. Naloxone.

29-Antabuse: {kasr}

a. Causes nausea, vomiting, sweating& palpitation.

b. Blocks the oxidation of acetaldehyde.

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c. Makes the addict hate drinking alcohol. d. All of the above.

30-Withdrawal of amphetamine, causes all of the following, except: {kasr}

a. Fatigue b. Insomnia. c. Depression. d. Confusion.

31-Interaction between disulfiram & ethanol results in{kasr}

a. Increased excretion. b. Increased oxidation.

c. Preventing oxidation. d. Preventing excretion.

32-Amphetamine is a strong CNS stimulant. {kasr}

a. true b. false

33-There is a continuous desire to take the abused drug by the addict. . {kasr}

a. true b. false

34-The addict maintains the same dose of the abused drug for long periods. . {kasr}

a. true b. false

35-Drug habituation is characterized by withdrawal symptoms. . {kasr}

a. true b. false

36-Clonidine can be used in treatment of opiate addicts. . {kasr}

a. true b. false

37-Naltrexone is given to addicts who stopped morphine. . {kasr}

a. true b. false

38-The addict needs drug in order to be in a normal psychological and mood status with feeling of satisfaction, that means

A. Drug dependence B. Tolerance.

C. Physical dependence D. Psychological dependence

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39-Tolerance is

A. Increase in a pharmacological effect on repeated administration of the drug, need to increase the dose to reach MORE pharmacological effect.

B. Decrease in a pharmacological effect on repeated administration of the drug, need to decrease the dose to reach the pharmacological effect of the same intesity.

C. decrease in a pharmacological effect on repeated administration of the drug, need to increase the dose to reach the pharmacological effect of the same intensity.

D. Increase in a pharmacological effect on repeated administration of the drug, need to increase the dose to reach the pharmacological effect of the same intesity.

40-Withdrawal of opiate includes the following except:

a. Increased body secretions b. Increased pulse rate c. Hypotension d. yawing

41-In addict: developing delusion, misleading hallucination of sight and hearing and the feeling of creeping things 'grains of sand' in the skin he is addict to

a. methadone b. heroin c. amphetamines d. cocaine

42-Naltrexone can be used in treatment of alcohol dependence to:

a. inhibit alcohol dehydrogenase enzyme.

b. Prevent opiate receptor mediated rewarding effects of alcohol.

c. Decrease autonomic hyperactivity.

d. Decrease alcohol metabolism.

43-Withdrawal manifestation of opiate addiction do not include

a. Running nose, tearing eyes. b. Sweats, nausea, diarrhea,

c. Joint pains. d. Seizures.

44-As regard disulfiram the following sentence is true:

a. It is used in treatment of barbiturate dependence.

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b. It inhibits alcohol aldehyde dehydrogenase enzyme.

c. It leads to bradycardia, hypertension and seizures.

d. It stops alcohol metabolism at formaldehyde stage.

45-Pathological jealousy syndrome is diagnostic of:

a. Nicotine intoxication. b. Alcohol intoxication.

c. Cocaine intoxication. d. Cannabis intoxication.

46-As regards Disulfiram, the following statement is false:

a. It stops the alcohol metabolism at formaldehyde stage.

b. It inhibits alcohol aldehyde dehydrogenase enzyme.

c. It leads to tachycardia, hypotension and feeling of general distress.

d. It stops the alcohol metabolism at acetaldehyde stage

47-The withdrawal manifestations in an addict are due to:

a. Psychological dependence. b. Physical dependence. c. Tolerance

d. Overdosage e. None of the above.

48-Drinking of ethanol is a patient under disulfiram therapy will lead to accumulation of:

a. Formic acid. b. Acetaldehyde. c. Carbon dioxide

d. Acetic acid e. Formaldehyde.

49-The drug of choice in treatment of chronic alcoholism is:

a. Diazepam b. Methadone. c. Codeine

d. Hyoscine e. Antabuse

50-A person under treatment with antabuse & ingests an alcoholic beverage will suffer from:

a. Tremors & convulsions. b. Bradycardia & hypotension. c. Thirst & fever.

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d. Stupor then coma e. Tachycardia & vomiting.

51-The following drug is useful in withdrawal treatment of morphine addiction:

a. Naloxone. b. Methadone. c. Nallorphine.

d. Flumazenil. e. Nalmefen

52-A male person with pale face, perforated nasal septum & dilated pupils is suffering from:

a. Mercurialism. b. Chronic arsenical poisoning. c. Heroin dependence.

d. Cocaine dependence. e. Plumbism.

53-Sudden withdrawal is better to be avoided in cases of:

a. Chronic alcohol abuse. b. Chronic cocaine abuse.

c. Chronic amphetamine abuse. d. Morphine addiction.

e. None of the above.

54-Sudden withdrawal is better to be avoided in cases of:

a. Chronic alcohol abuse. b. Chronic cocaine abuse. c. Chronic amphetamine abuse..

d. Barbiturates addiction. e. None of the above.

55-Sudden withdrawal is better in cases of:

a. Chronic alcohol abuse b. Chronic cocaine abuse. c. Chronic amphetamine abuse.

d. All of the above. e. None of the above.

56-Gradual withdrawal is not better in cases of:

a. Barbiturates addiction. b. Morphine addiction

c. Chronic amphetamine abuse. d. All of the above. e. None of the above.

57-Gradual withdrawal is not better in cases of:

a. Barbiturates addiction. b. Morphine addiction. c. Chronic cocaine abuse.

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d. All of the above. e. None of the above.

58-Gradual withdrawal is not better in cases of:

a. Barbiturates addiction. b. Morphine addiction. c. Chronic alcohol abuse.

d. All of the above. e. None of the above

1. A	2. D	3. D	4. B	5. A
6. C	7. A	8. A	9. C	10.A
11.C	12.B	13.C	14.A	15.C
16.C	17.B	18.C	19.B	20.C
21.B	22.D	23.B	24.C	25.D
26.B	27.C	28.A	29.D	30.B
31.C	32.A	33.A	34.B	35.B
36.A	37.A	38.D	39.C	40.C
41.D	42.B	43.D	44.B	45.B
46.A	47.B	48.B	49.E	50.E
51.B	52.D	53.D	54.D	55.D
56.C	57.C	58.C	59.	60.

Pesticides

1-Cause of death in organo phosphorous poisoning is {Final2025}

A-Laryngeal edema B-Respiratory muscle spasm C-Decreased bronchial secretions

2- Delayed Neuropathy due to organophosphorus poisoning is; {Mid2021}{Formative2025}

a. Motor b. Sensory c. Mixed motor and sensory d. None of them

3-The antidote for Organophosphorus is: {Formative2022}

a) digibind b) N. Acetyl cysteine c) atropine d) defroxamine

4-All the following are muscarinic effects of organophosphorus poisoning except: {Formative2025}

a)Muscle fasciculation b)Bradycardia c)bronchoconstriction d)Urination

5- Organophosphate intermediate syndrome? {Mid2025}{kasr}

A-Developed within 1-5 weeks B-Cause paralysis of distal limb muscles

C-Cause neck flexion paralysis D-Death is due to circulatory collapse

6- In organophosphorus poisoning, Intermediate syndrome occurs due to {Mid2024}

A. Inadequate atropine therapy B. Inadequate oxime therapy

C. Inadequate oxygen therapy D. Atropine overdose

7- Organophosphate intermediate syndrome is {Final2022}

A- Characterized by weakness of distal limb muscles.

B- Due to inadequate atropine therapy

C- Necrotizing myopathy begins shortly after the initial decline in true cholinestrase enzyme activity.

D- Its intensity is not correlated with the degree of fall of the enzyme.

8- All of the following are muscarinic manifestation of organophosphorus EXCEPT {Final2023}

A- Muscle fasciculation B- vomiting C- urination D- bronchoconstriction

9- Atropine is used in treatment of organophosphate poisoning until all of the following EXCEPT. {Final2022}

A- Chest is clear. B- Full atropinization C- Relieve bronchospasm. D- Muscle relax.

10- Delayed neuropathy of organophosphorus is {Formative2023}{Final2023}

A- motor B- sensory C- Mixed motor and sensory d- Not permanent

11-Oximes are used as antidote in carbamate poisoning? {Final2025}

A-True B-False

12-Oximes are: {قديم}{Formative2023}

a. Cholinesterase reactivators b. Cholinesterase inhibitors

c. Antidote for carbamates poisoning d. None of the above

13-Life saving antidote of Organophosphorus poisoning is oxime {Formative2025}

a) True b) False

14-Organophosphorus compound can be absorbed by intact skin? {Mid2025}

A- true B- false

15-Carbamates are more toxic than Organophosphorus on human health: {Formative2022}

a) True b) False

16- Zinc phosphide is highly toxic because it releases…….on contact with water or gastric acids {Final2022}

A- Arsine gas B- Phosphine gas C- H2S D- Cyanide gas

17- Rodenticide black in color with a rotten fish Oduor and taste {Mid2024}

A. Red squill B. Warfarin C. Zinc phosphide D. Strychnine sulphate

18-Zinc phosphide causes its action through the release of: {Formative2024}

A. Arsine gas B. Phosphine gas C. H2S D. Cyanide gas

19-Intermediate syndrome occurs due to: {Final2021}

a. Inadequate atropine therapy b. Atropine overdose

c. Inadequate oxime therapy d. Inadequate oxygen therapy

20- For treatment of carbamate poisoning, we can use: {Final2021}

a. Atropine b. Naloxone c. Oxime d. a and c

21-Which one of the following is considered as an antidote for organophosphate poisoning

A. n-acetylcysteine B. Ethanol/fomepizole C. Oximes D. Naloxone/nalmefene

22-Organophosphorous antidote is {Merit}

a. Digibind b. Mephensen c. Atropine d. Naloxone

23- All of the following are rodenticides except {قديم}

A- thalium B- yellow phosphorus C- warfarin D- paraquat

24-All the following are nicotinic manifestations of organophosphate poisoning except: {Qena}

a. motor weakness. b. Muscle fasciculation. c. convulsions. d. miosis

25-Manifestations of organophosphate poisoning include all the following except: {Qena}

a. Diarrhea. b. Medriasis. c. Lacrimation. d. Excessive salivation.

26- Oximes are recommended in treatment of carbamate toxicity {قديم}

A- true B- False

27- Oximes must be used early in organophosphate toxicity {قديم}

A- true B- False

28-Patient with organophosphrous poisoning presented by pulmonary edema and respiratory muscle weakness. This situation is best managed by: {Qena}

a. Readministration of atropine b. Readministration of atropine + Oximes

c. Readministration of Oximes d. Readministration of atropine + 100% oxygen

29-The patient with SLUD syndrome, miosis & convulsions suffers from poisoning with: {kasr}

a. Parathion. b. Carbamate. c. Parasympathetic d. All of the above.

30-Phenol, opium, organophosphorus and cyanide: {kasr}

a. Cause miosis. b. Cause mydriasis.

c. Have characteristics smell. d. Have corrosive action.

31-Carbamate poisoning is characterized by all the following, except: {kasr}

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a. Bradycardia. b. Increased secretions.

c. Nicotinic manifestations. d. Mydriasis

32-Atropine can be used in treatment of all the following, except: {kasr}

a. Organophosphorus poisoning. b. Carbamate insecticides.

c. B-adrenergic blockers poisoning d. Cyanid toxicity.

33-All the following poisonings can be detected after putrefaction, except: {kasr}

a. Orgaonphosphorus. b. Snake venom. c. Cadmium. d. Arsenic.

34-Organophosphorus poisoning is characterized by all the following, except: {kasr}

a. Tachycardia. b. Increase intestinal sounds.

c. Pin point pupil. d. Garlic smell.

35-Pyrethrins cause: {kasr}

a. Cutaneous and inhalational allergy.

b. Prolong nerve sodium channel currents in insects.

c. Tremors, paralysis or seizures.

d. All of the above.

e. None of the above.

36-Respiratory failure in O.P poisoning is due to {kasr}

a. Decreased bronchial secretions. b. Laryngeal edema.

c. Respiratory muscle spasm.

d. Distal degeneration of peripheral and spinal cord nerves.

37-Bradycardia and heart block in O.P is due to: {kasr}

a. Nicotine like action. b. Muscarinic like action.

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c. Central action. d. All of the above

38-All are signs of parathion poisoning, except: {kasr}

a. Tachycardia b. Headache. c. Bronchospasm d. Diarrhea

39-In treatment of organophosphorus atropine treat all of the following, except: {kasr}

a. It doesn,t restore Choline esterase activity.

b. It antagonizes nicotinic effect of O.P.

c. It antagonizes muscarinic effect of O.P. d. Cross blood brain barrier.

40-Atropine is used in treatment of organophosphorus poisoning until all of the Following, except: {kasr}

a. Chest is clear. b. Full atropinization.

c. Correction of bradycardia. d. Muscles relax.

41-Organophosphorus poisoning most commonly homicidal. {kasr}

a. true b. false

42-25% depression in cholinesterase level in blood is indicator of poisoning. {kasr}

a. true b. false

43-Fatigue, fasciculations and paralysis are symptoms of O.P poisoning due to muscarinic like action. {kasr}

a. true b. false

44-Atropine sulfate is a pure antagonist to acetylcholine at muscarinic receptors. {kasr}

a. true b. false

45-Prophylactic decontamination of clothes, skin and utensils contaminated by O.P is the most important issue in O.P management. {kasr}

a. true b. false

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46-Oximes must be given in first 24-48 hours after O.P poisoning. {kasr}

a. true b. false

47-Organophosphorus can be absorbed through intact skin. {kasr}

a. true b. false

48-Oximes are used in treatment of carbamate insecticides. {kasr}

a. true b. false

49-Atropine must be taken with pralidoxime (PAM) in treatment O.P toxicity. {kasr}

a. true b. false

50-Oximes can be taken at any time after O.P poisoning. {kasr}

a. true b. false

51-Asphyxia in O.P is due to of O.P only. {kasr}

a. true b. false

52-Atropine sulfate has important role in treatment of delayed post organophosphorus poisoning neurological complications. {kasr}

a. true b. false

53-In organophosphorous toxicity, the pupil examination revealed .... {Merit}

a. Dilated reactive b. Unchanged c. Dilated fixed d. Miosis

54-Endpoint of atropine therapy in organophosphorus poisoning: {Qena}

a.Dilated pupils. b.Tachycardia. c.Increased temperature. d.Dry bronchial secretions.

55-Asphyxia in organophosphorus poisoning is due to the following except: {Qena}

a. Paralysis of respiratory muscles b. Depression of respiratory center

c. Bronchospasm and bronchial secretion d. Edema of larynex

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56-organophosphorus poisoning characterized by the following except: {Qena}

a-garlic smell. b-increased secretions. c-pinpoint pupil. d-deep profound coma

57- Give atropine in organophosphorus until {Qena}

a) Chest dryness b) Vomiting stop c) The pupil dilate d) Heart rate increase

58-Regarding toxicity by Carbamates, the following are true except: {Qena}

a- Onset of symptoms may be more rapid that with OPC.

b- Recovery is rapid. Poisoning seldom exceeds 2 days.

c- Pseudo-cholinesterase enzyme is replenished within few days.

d- No intermediate syndrome, delayed neuropathy or cardiotoxic effect.

59-Carbamates differ from organophosphates in the following:

a) Inhibition of cholinesterase enzyme. b) Atropine is the physiological antidote.

c) Penetration of blood brain barrier. d) Bradycardia and hypotension.

60-Garlic breath odour can be found in patients with which of the following poisoning:

a. Organophosphates. b. Cyanide. c. Aspirin. d. Atropine.

61-Organophosphorus insecticides inhibit:

a. Cytochrome oxidase enzyme b. Cholinestenese enzyme.

c. Sulph-hydryl enzymes. d. Dehydrogenase enzyme.

e. Pyrimidine-5-nucleotidase enzyme

62-Organophosphate poisoning can cause convulsions by

a. Direct effect at myoneural junction. b. Potentiation of parasympathetic activity.

c. Central nervous system stimulation. d. Spinal cord stimulation,

63-Nicotinic effects of organophosphate poisoning include all the following:

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a. Tachycardia b. Pulmonary edema. c. Cyanosis.

d. Hypertension. e. a and d.

64-Intermediate syndrome may occur in organophosphorus poisoning due to:

a. Inadequate atropine therapy. b. Inadequate oxygen therapy.

c. Inadequate oxime therapy d. Fat soluble organophosphorus compounds.

e. Neurotoxic organophosphorus compounds.

65-Contact dermatitis is the most common manifestation in poisoning with:

a. Parathion. b. Warfarin. c. Dinitrophenol.

d. Zinc phosphide. e. Pyrethrins.

66-Severe anaphylactic reaction can occur in poisoning with:

a. Temik. b. Red squill. c. Ezalo. d. Dinitrophenol.

67-Tachycardia can occur in organophosphorus poisoning due to:

a. Inhibition of cholinesterase enzyme. b. Nicotinic action.

c. Muscarinic action. d. CNS stimulation. e. None of the above.

68-Increase in all secretions is a constant feature of the acute poisoning by:

a. Naphthalene. b. Pyrethroids. c. Parathion. d. Zinc phosphide.

69-The acute toxicity of anticholinesterases produces:

a. Urinary retention. b. Urinary incontinence. c. Oliguria. d. Polyuria.

70-Bronchospasm in organophosphorus poisoning is due to the following action:

a. Nicotinic. b. Central. c. Muscarinic

d. Direct on smooth muscles. e. Allergic.

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71-Muscle twitches then paralysis, in severe cases of organophosphorus poisoning, are due to action on:

a. Muscarinic receptors. b. Central nervous system.

c. Nicotinic receptors. d. Muscles.

72-Pralidoxime:

a. Reactivate cholinesterase. b. Is antidote for parathion. c. Is given I.V

d. The adult dose is 1 -2gm. e. All of the above.

73-The followings are lines for treatment of naphthalene poisoning Except:

a. Decontamination. b. Alkalinization of urine. c. Fresh blood transfusion.

d. Diet high in fat. e. Hemodialysis.

74-A rodenticide which is black in colour & has a rotten fish odour:

a. Temik. b. Zinc phosphide. c. Red squill. d. Warfarin.

75-Oximes are not indicated in cases of acute poisoning with:

a. Zink phosphide. b. Parathion. c. Carbamates.

d. All of the above. e. a and c

76-The followings are toxic effects of naphthalene Except:

a. CNS stimulation then depression. b. Hemolysis of RBCs.

c. Hepatic necrosis d. Dermatitis. e. Pulmonary fibrosis.

77-the following is a muscarinic manifestations of parathion:

a. Tachycardia. b. Bronchospasm. c. Muscular fasciculations.

d. Convulsions. e. Hypertension

78-atropine is used as antidote for acute parathion poisoning as it:

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a. Control muscular fasciculations. b. Blocks muscarinic action.

c. Blocks effect on autonomic ganglion. d. All of the above. e. a & c only

79-Organonitrogenous insecticides characterized by the following:

a. Inactivate cholinesterase by thiol radical.

b. Cause more CNS toxicity than in case organophosphorus.

c. Oximes are the only antidote.

d. Symptoms of poisoning are of shorter duration than in case organophosphorus.

e. Atropine is contraindicated in their treatment.

80-in Zinc phosphide toxicity

(a) gastric lavage should be done with tape water

(b) patient presented firstly with Hepatotoxicity

(c) it causes metabolic alkalosis

(D) toxicity mainly is due to release of phosphine gas

81-With regards to organophosphates and carbamates which is false?

A. Both respond to treatment with atropine. B. both inactivate acetyl cholinesterase enzyme.

C. both require treatment with pralidoxime. D. both cause an acute cholinergic syndrome

82-Organophosphorus compounds inhibit

A) Acetylcholine synthesis B) Acetylcholine esterase enzyme

C) Acetylcholine release D) Acetylcholine activation

83-carbamate toxicity

(a) due to irreversible inhibition of choline esterase enzyme

(b) presented with muscle fasciculations and pin point pupil

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(c) treatment with oxime is a must

(d) treatment is mainly with atropine and oxime

84-……is not seen in organophosphate poisoning

A) Lacrimation B) Salivation C) Mydriasis D) Diarrhoea

85-The drug used for cholinesterase enzyme reactivation in organophosphate poisoning is

A) Atropine B) activated charcoal C) Sodium bicarbonate D) obidoxime

86-A 28 year-old man presented to poison control center with rapid onset of severe watery diarrhea, vomiting and abdominal pain, hypersalivation, pinpoint pupil, bradycardia and garlic odour. This scenario is MOST consistent with acute poisoning with?

A. Arsenic. B. Salicylate. C. Paracetamol. D. An Organophosphate pesticide

87-male patient 30 years old presented with vomiting ,diarrhea ,sweating , pin point pupil and musclefasciculations the best treatment is

(a) Atropine (b) naloxone (c) Atropine and oxime (d) IV fluid

88-A patient presented with the following symptoms and sign: diarrhea, urination, miosis, bradycardia, bronchospasm, sweating. It is mostly represents?

A. Opioid toxicity. B. Atropine toxicity

C. Organophosphate poisoning. D. Amphetamine toxicity

89-Regarding carbamate poisoning

A. Carbamates bind irreversibly to acetyl cholinesterase enzyme.

B. Carbamates have a shorter duration of action than organophosphates.

C. Pralidoxime is mainstay in treatment.

D. Atropine alone is ineffective in treatment in most cases

90-The main cause of death in organophosphate poisoning is secondary to

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A. Respiratory failure. B. Renal failure. C. Hepatotoxicity. D. Cardiotoxicity.

91-Muscarinic manifestations of organophosphates are treated with:

A) Atropine. B) Nicotine. C) Morphine. D) Naloxone.

1. B	2. C	3. C	4. A	5. C
6. B	7. C	8. A	9. D	10.C
11.B	12.A	13.B	14.A	15.B
16.B	17.C	18.B	19.C	20.A
21.C	22.C	23.D	24.D	25.B
26.B	27.A	28.B	29.D	30.C
31.D	32.D	33.A	34.A	35.D
36.C	37.B	38.A	39.B	40.D
41.B	42.B	43.B	44.A	45.A
46.A	47.A	48.B	49.A	50.B
51.B	52.B	53.D	54.D	55.D
56.D	57.A	58.C	59.C	60.A
61.B	62.C	63.E	64.C	65.E
66.C	67.B	68.C	69.B	70.C
71.C	72.E	73.D	74.B	75.E
76.E	77.B	78.B	79.D	80.D
81.C	82.B	83.B	84.C	85.D
86.D	87.C	88.C	89.B	90.A
91.A	92.	93.	94.	95.

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Animal poisons

1- Cobra toxin is {Mid2024}

A. Neurotoxin B. Cardiotoxin C. Myotoxin D. Vasclutoxin

2-Factors affecting the severity of the symptoms of a snake bite include: {Formative2024}

a) Depth of bite b) Species of snake c) Age of the victim d) All of the above

3-. Toxic manifestation of scorpion sting includes the following EXCEPT {Final2021}

a. Hypertension b. Pulmonary edema c. Bleeding tendency d. Vomiting

4- The main component of scorpion venom {Final2022}{Mid2022}{Formative2024}

A- Histamine B- Serotonin C- Neurotoxin D- Cardiotoxin

5-Tourniquets is indicated in treatment of snake bite {Formative2022}

a) True b) False

6- A 30 year old male patient came to hospital suffering from snake bite by examination the doctor found muscle weakness and the patient known to snake hypersensitivity have antivenin which of the following is specific treatment? {Final2022}

A- Observation and local analgesic B- Antivenin with antibiotics

C- Antivenin, suction and compression of the site bite

D- Antivenin, adrenaline and antihistamine

7-Priapism is a character of {Merit}{Qena}

a. Spider sting b. Snake bite c. Paralytic shell fish contacting d. Scorpion sting

8- All of the following is indicated in snake bite treatment except: {قديم}{Qena}

A- Rest and reassurance B- Immobilization C- Antibiotic and steroid D- Ice pack

9-Application of incision to the site of snake bite is a safe procedure. {Qena}

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a. True b. False

4-The commonest manifestation of viper bite is. {Qena}

a- Local tissue necrosis. b- Shock. c- Dysphagia and parathesia. d- Non of the above.

10-risk factor in snake bite ? {Qena}

a. Biting in head

11-Viper venom produces: {kasr}

a. Mild local injury. b. Paralysis. c. Renal failure. d. Dysphagia.

12-All of the following is true regarding treatment of snake bite, except: {kasr}

a. Antivenin neutralizes the venom, but don't reverse local injury.

b. Transfer the victim immediately to the hospital.

c. Tetanus vaccination is contraindicated.

d. Antivenom are suspensions of venom-neutralizing antibodies derived from the sera of hyperimmunized animals (horses)

13-All of the following is true regarding scorpions, except: {kasr}

a. They carry their venom in their tails.

b. They are not generally aggressive towards humans

c. The site of scorpion sting can be easily seen.

d. There is no commercially available antivenin

14-All of the following is true regarding snake venom, except: {kasr}

a. It functions as part of the snake's food gathering mechanism, and defensive mechanism.

b. Neurotoxins are neuro-muscular non-depolarizing blocking agents

c. Enzymatic components provide lethality.

d. Venom components and activity can vary according to several factors.

15-Regarding treatment of snake bite, all of the following is true, except: {kasr}

a. Suction by mouth or suction apparatus is not effective in removing significant amount of venom.

b. Ice pack may result in serious frost bite.

c. Adults require more antivenin than children.

d. Antivenin is the treatment of choice for coagulopathies.

16-Hyperglycemia occurs in poisoning with{kasr}

a. Scorpion venom b. Ethanol c. cannabis d. Methanol

17-Application of a tourniquet to the site of snake bite is a safe procedure. {kasr}

a. true b. false

18-The presence of fang marks is indicative of envenomation. {kasr}

a. true b. false

19-One of the earliest neurotoxic manifestation in elapid snake bite is

a. staggering gait. b. Difficult swallowing c. Bilateral ptosis d. Drooling of saliva

20-in Viperidea snake bite, one of the following statements is false

a. Lethargy, staggering gait and ptosis are uncommon findings.

b. Hemoglobinuria is very common.

c. Ecchymosis, redness and blisters are common

d. Circulatory collapse and shock are rarely found.

21-Which of the following antidotes is considered as antibodies?

a. Antibotulism antitoxin. b. Snake antivenom. c. Scorpion antivenom.

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d. FAB fragment. e. All of the above.

22-Elapidae Snakes include:

a) Cobra snakes. b) Sea snakes. c) Viper snakes.

d) Pit vipers. e) Mangrove snakes.

23-Cobra is one of:

a) Elapidae snakes. b) Hydrophidae snakes. c) Viperidae snakes.

d) Crotalidae snakes e) Colubridae snakes.

24-Egyptian Cerastes viper is one of:

a) Elapidae snakes. b) Hydrophidae snakes. c) Viperidae snakes.

d) Boidae snakes. e) Colubridae snakes.

25-A bite from Elapidae snake is characterized by the following Except

a) Giddiness, lethargy and muscular weakness b) Salivation, nausea and vomiting.

c) Loss of consiousness. d) Ptosis.

26-A bite from viper snakes is characterised by the following Except:

a) Hypotension & shock. b) Nausea and vomiting. c) Minimal local symptoms.

d) Petechial hemorrhage and bleeding from the body orifices.

e) Haemoglobinuria due to intravascular hemolysis

27-Antivenom is characterised by:

a) It is the only specific treatment for poisoning with snake venom.

b) it is serum of horses immunized with the venom c) It is adminestered by I.V. dripping.

d) All of the above.

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1. A	2. D	3. C	4. C	5. B
6. D	7. D	8. D	9. B	10.A
11.A	12.C	13.C	14.C	15.C
16.A	17.B	18.B	19.C	20.D
21.E	22.A	23.A	24.C	25.B
26.C	27.D	28.	29.	30.

Food poisoning

1-ptosis, difficulty swallowing and paralysis are clinical picture of {Formative2022}

a) Botulism poisoning b) Staph poisoning

c) Salmonella poisoning d) None of the above

2- All of the following are manifestations of botulism EXCEPT {Final2023}{Qena}

A- diarrhea B- Muscle paralysis C- Urine retension D- Fixed dilated pupil

3- Botulism toxicity causes the following except {قديم}

A- Blurring of vision B- Dysphagia C- Convulsion D- Muscle weakness

4-The neurological effect of botulism is: {Qena}

A. Ascending paralysis B. Descending paralysis C. Seizures D. Tremors

5-Cases of botulism present with the following except: {Qena}

a-Dry mouth b-convulsion c-Abdominal pain d- Constipation

6-Ptosis occurs in poisoning by: {kasr}

a. Botulism. b. Lead. c. Parathion. d. Amitryptyline.

7-All of the following is true regarding botulism, except: {kasr}

a. Botulinum toxin is one of the most powerful & lethal toxins known.

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b. The toxin is heat stable

c. The toxin blocks the release of acetylcholine.

d. Gastrointestinal symptoms do not occur in approximately 1/3 of patients.

8-Botulinum toxin: {kasr}

a. Produces miosis. b. Is treated by oximes.

c. Is primarily hemolytic d. Produces constipation in many cases.

9-Staphylococcal food poisoning is characterized by: {kasr}

a. Offensive stools. b. Fever. c. Delayed onset. d. The toxin is heat stable

10-All of the following is true regarding bacterial food poisoning, except: {kasr}

a. Staph. aureus usually present in meat, dairy products, and bakery.

b. It starts after incubation period which is usually short (4-6 hours) after invasive type.

c. The toxin of Staph. aureus is heat stable.

d. Constitutional symptoms are common manifestations of enteroinvasive types of food poisoning.

11-All of the following is true regarding food poisoning, except: {kasr}

a. The onset of symptoms and signs occurs in many persons at the same time.

b. May be due to contamination by biological agents.

c. Hypersensitivity reactions e.g. urticaria, bronchial asthma may occur on ingestion of certain foods as strawberries, fishes and eggs.

d. Diarrhea is profuse, watery and severe following Salmonella food poisoning.

12-All of the following is true regarding investigations of food poisoning, except: {kasr}

a. Stool culture may be diagnostic and of epidemiological concern.

b. Decrease of hematocrit denotes moderate to severe dehydration

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c. Fecal leukocytes denotes microbial invasion of the gut.

d. Complete blood count (CBC) may vary from mild stress-related leukocytosis (toxigenic) to severe leukocytosis in enteroinvasive types.

13-Anticholinergic finding (s) in botulism occur (s) in the form of: {kasr}

a. Fever. b. Dry mouth. c. Rapid pulse. d. All of the above

14-The muscle weakness of botulism characteristically starts in: {kasr}

a. The muscles supplied by the cranial nerves. b. The muscles of the upper limb

c. The respiratory muscles. d. The muscles of the lower limb.

15-Botulism is a clinical syndrome that results only from ingestion of botulinum toxin. {kasr}

a. true b. false

16-Clostridium botulinum is an aerobic spore forming gram -ve rod which contains 7 antigenically distinct serologic types. {kasr}

a. true b. false

17-Botulinum toxin blocks the synthesis of acetylcholine. {kasr}

a. true b. false

18-In cases of botulism, sluggish pupillary reactivity indicates a higher incidence of respiratory failure. {kasr}

a. true b. false

19-One of the following signs is absent in botulism

a. Intact mentation b. cardiac arrest

c. Reduced reflexes d. ascending muscle weakness

20-In cases of food-borne botulism the toxin is formed

a. In the canned food. b. In the intestine.

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c. In the duodenum. d. In the colon.

21-Honey can cause:

a) Wound botulism. b) Food borne botulism.

c) Infant botulism. d) Inadvertent botulism.

22-Incubation period of wound botulism is:

a. Within 2 weeks. b. Within 3 weeks. c. Within 4 weeks. d. Within 5 weeks.

23-Which of the followings are the commonest types of human clostridiuan botulinum neurotoxin?

a. A, B, and E b. D, E, F, and G. c. C- D and E

d. A, B, F, and G. e. B, C, F, and G.

24-Botulism includes the following types:

a. Food-borne botulism, infant botulism and wound botulism..

b. Infant botulism and wound botulism. c. Food-borne botulism and infant botulism

d. Food-borne botulism e. Wound botulism.

25-The clinical picture of food-borne botulism includes the following:

a. Initial gastro-intestinal symptoms of food poisoning. b. Affection of cranial nerves.

c. Descending symmetrical paralysis with intact mentation. d. All of the above.

e. None of the above.

26-Which of the following is the specific antidote of botulism:

a. Antibotuliam antitoxin. b. Snake antivenom. d. FAB fragment.

c. Scorpion antivenom. e. All of the above

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1. A	2. A	3. C	4. B	5. B
6. A	7. B	8. D	9. D	10.B
11.D	12.B	13.B	14.A	15.B
16.B	17.B	18.A	19.D	20.A
21.C	22.A	23.A	24.A	25.D
26.A	27.	28.	29.	30.